Application Submitted to HRSA

Submitted to HRSA

Organization: UNIVERSITY OF MONTANA, MISSOULA, Montana

Grants.gov Tracking Number: GRANT14708695

EHB Application Number: 247915

Grant Number: N/A

Funding Opportunity Number: HRSA-26-019

Received Date: 7/10/2026 3:15:45 PM

A printer version document only. The document may contain some accessibility challenges for the screen reader users. To access same information, a fully 508 compliant accessible HTML version is available on the HRSA Electronic Handbooks. If you need more information, please contact HRSA contact center at 877-464-4772, 8 am to 8 pm ET, weekdays.

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OMB Number: 4040-0001
Expiration Date: 01/31/2029

APPLICATION FOR FEDERAL ASSISTANCE

SF 424 (R&R)

3. DATE RECEIVED BY STATE

State Application Identifier

1. TYPE OF SUBMISSION

☐ Pre-application ☒ Application ☐ Changed/Corrected Application

4. a. Federal Identifier

b. Agency Routing Identifier

c. Previous Grants.gov Tracking ID

2. DATE SUBMITTED

Applicant Identifier

5. APPLICANT INFORMATION

UEI: DAY7Z8ZD48Q3

Legal Name: University of Montana

Department:

Division:

Street1: 32 Campus Drive

Street2:

City: Missoula County / Parish: MT

State: MT: Montana Province:

Country: USA: UNITED STATES ZIP / Postal Code: 59812-0003

Person to be contacted on matters involving this application

Prefix: First Name: Emery Middle Name:

Last Name: Reyna Suffix:

Position/Title: Proposal Development Analyst

Street1: 32 Campus Drive

Street2:

City: Missoula County / Parish:

State: MT: Montana Province:

Country: USA: UNITED STATES ZIP / Postal Code: 59812-0003

Phone Number: 406-243-4307 Fax Number:

Email: emery.reyna@umontana.edu

6. EMPLOYER IDENTIFICATION *(EIN) or (TIN):* 81-6001713

7. TYPE OF APPLICANT: H: Public/State Controlled Institution of Higher Education

Other (Specify):

Small Business Organization Type ☐ Women Owned ☐ Socially and Economically Disadvantaged

8. TYPE OF APPLICATION:

☒ New ☐ Resubmission

☐ Renewal ☐ Continuation ☐ Revision

If Revision, mark appropriate box(es).

☐ A. Increase Award ☐ B. Decrease Award ☐ C. Increase Duration ☐ D. Decrease Duration

☐ E. Other (specify):

Is this application being submitted to other agencies? Yes ☐ No ☒ What other Agencies?

9. NAME OF FEDERAL AGENCY:

Health Resources and Services Administration

10. ASSISTANCE LISTING NUMBER: 93.877

ASSISTANCE LISTING TITLE:

Autism Collaboration, Accountability, Research, Education, and Support

11. DESCRIPTIVE TITLE OF APPLICANT'S PROJECT:

The Montana Leadership Education in Neurodevelopmental and Other Related Disabilities Program

12. PROPOSED PROJECT:

Start Date: 09/30/2026

Ending Date: 06/30/2031

13. CONGRESSIONAL DISTRICT OF APPLICANT

MT-001

14. PROJECT DIRECTOR/PRINCIPAL INVESTIGATOR CONTACT INFORMATION

Prefix: | First Name: Kimberly | Middle Name: Ann

Last Name: McKearnan | Suffix:

Position/Title: Program Director and Assistant Professor

Organization Name: University of Montana

Department: Office of Sponsored Programs

Division: Rural Institute

Street1: 32 Campus Drive #2304

Street2:

City: Missoula | County / Parish: MT

State: MT: Montana | Province:

Country: USA: UNITED STATES | ZIP / Postal Code: 59812-0003

Phone Number: 406-243-6930 | Fax Number:

Email: kim.mckearnan@mso.umt.edu

15. ESTIMATED PROJECT FUNDING

a. Total Federal Funds Requested	2,298,597.00
b. Total Non-Federal Funds	0.00
c. Total Federal & Non-Federal Funds	2,298,597.00
d. Estimated Program Income	0.00

16. IS APPLICATION SUBJECT TO REVIEW BY STATE EXECUTIVE ORDER 12372 PROCESS?

a. YES ☐ THIS PREAPPLICATION/APPLICATION WAS MADE AVAILABLE TO THE STATE EXECUTIVE ORDER 12372 PROCESS FOR REVIEW ON:

DATE:

b. NO ☒ PROGRAM IS NOT COVERED BY E.O. 12372; OR

☐ PROGRAM HAS NOT BEEN SELECTED BY STATE FOR REVIEW

17. By signing this application, I certify (1) to the statements contained in the list of certifications* and (2) that the statements herein are true, complete and accurate to the best of my knowledge. I also provide the required assurances * and agree to comply with any resulting terms if I accept an award. I am aware that any false, fictitious. or fraudulent statements or claims may subject me to criminal, civil, or administrative penalties. (U.S. Code, Title 18, Section 1001)

☒ **I agree**

**The list of certifications and assurances, or an Internet site where you may obtain this list, is contained in the announcement or agency specific instructions.*

18. SFLLL (Disclosure of Lobbying Activities) or other Explanatory Documentation

Add Attachment | Delete Attachment | View Attachment

19. Authorized Representative

Prefix: | First Name: Nicole | Middle Name:

Last Name: Thompson | Suffix:

Position/Title: Director, Office of Sponsored Programs

Organization: University of Montana

Department:

Division:

Street1: 32 Campus Drive

Street2:

City: Missoula | County / Parish: MT

State: MT: Montana | Province:

Country: USA: UNITED STATES | ZIP / Postal Code: 59812-0003

Phone Number: 406-243-6670 | Fax Number:

Email: awards@umontana.edu

Signature of Authorized Representative	Date Signed
Emery Reyna	07/10/2026

20. Pre-application Add Attachment | Delete Attachment | View Attachment

21. Cover Letter Attachment Add Attachment | Delete Attachment | View Attachment

The Montana Leadership Education in Neurodevelopmental and Other Related Disabilities (Montana LEND) program, proposed for launch in Fall 2026, will be the first standalone LEND program in Montana. This program will help address the longstanding need for a highly-trained workforce ready to serve Montanans with Autism and other developmental disabilities. Housed at the University of Montana's Rural Institute for Inclusive Communities, it will build on their established track record of interdisciplinary training, service, and research. Montana LEND aims to improve the health and quality of life of individuals with neurodevelopmental and related disabilities by preparing interdisciplinary leaders to deliver family-centered, culturally relevant, and coordinated care across the lifespan, with an emphasis on early identification and systems-level change. Beginning with at least nine trainees across multiple disciplines, including individuals with lived experience, Montana LEND will offer a comprehensive academic-year curriculum integrating didactic, clinical, and leadership training, with plans to add advanced clinical fellows by year three. A diverse faculty, including self-advocates and family members, will support training through in-person, virtual, and hybrid formats to ensure statewide access. The program will also provide ongoing continuing education and technical assistance to strengthen capacity and improve outcomes for individuals and families across Montana.

OMB Number: 4040-0019
Expiration Date: 06/30/2028

Project Abstract Summary

This Project Abstract Summary form must be submitted or the application will be considered incomplete. Ensure the Project Abstract field succinctly describes the project in plain language that the public can understand and use without the full proposal. Use 4,000 characters or less. Do not include personally identifiable, sensitive or proprietary information. Refer to Agency instructions for any additional Project Abstract field requirements. If the application is funded, your project abstract information (as submitted) will be made available to public websites and/or databases including USAspending.gov.

Funding Opportunity Number

HRSA-26-019

Assistance Listing Number(s):

93.877

Applicant Name

University of Montana

Descriptive Title of Applicant's Project

The Montana Leadership Education in Neurodevelopmental and Other Related Disabilities Program

Project Abstract

The Montana Leadership Education in Neurodevelopmental and Other Related Disabilities (Montana LEND) program, proposed for launch in Fall 2026, will be the first standalone LEND program in Montana. This program will help address the longstanding need for a highly-trained workforce ready to serve Montanans with autism and other developmental disabilities. Housed at the University of Montana’s Rural Institute for Inclusive Communities, it will build on their established track record of interdisciplinary training, service, and research. Montana LEND aims to improve the health and quality of life of individuals with neurodevelopmental and related disabilities by preparing interdisciplinary leaders to deliver family-centered, culturally relevant, and coordinated care across the lifespan, with an emphasis on early identification and systems-level change. Beginning with at least nine trainees across multiple disciplines, including individuals with lived experience, Montana LEND will offer a comprehensive academic-year curriculum integrating didactic, clinical, and leadership training, with plans to add advanced clinical fellows and an interdisciplinary evaluation and diagnostic clinic on campus by year three. A diverse faculty, including self-advocates and family members, will support training through in-person, virtual, and hybrid formats to ensure statewide access. The program will also provide ongoing continuing education and technical assistance to strengthen capacity and improve outcomes for individuals and families across Montana.

OMB Number: 4040-0001
Expiration Date: 01/31/2029

RESEARCH & RELATED Senior/Key Person Profile (Expanded)

PROFILE - Project Director/Principal Investigator

Prefix:
* First Name: Kimberly
Middle Name: Ann
* Last Name: McKearnan
Suffix:
Position/Title: Program Director and Assistant Professor
Department: Office of Sponsored Programs
Organization Name: University of Montana
Division: Rural Institute
* Street1: 32 Campus Drive #2304
Street2:
* City: Missoula
County/ Parish: MT
* State: MT: Montana
Province:
* Country: USA: UNITED STATES
* Zip / Postal Code: 59812-0003
* Phone Number: 406-243-6930
Fax Number:
* E-Mail: kim.mckearnan@mso.umt.edu

Credential, e.g., agency login: PhD

*** Project Role:** PD/PI
Other Project Role Category:

Degree Type: Nursing Science
Degree Year: 2004

***Attach Biographical Sketch**	1243-Kim_McKearnan Biosketch	Add Attachment	Delete Attachment	View Attachment
Attach Current & Pending Support		Add Attachment	Delete Attachment	View Attachment

PROFILE - Senior/Key Person 1

Prefix:
* First Name: Molly
Middle Name:
* Last Name: Kimmel
Suffix:
Position/Title: Director, Rural Institute
Department: Office of Sponsored Programs
Organization Name: University of Montana
Division: Rural Institute
* Street1: 32 Campus Drive #2304
Street2:
* City: Missoula
County/ Parish: MT
* State: MT: Montana
Province:
* Country: USA: UNITED STATES
* Zip / Postal Code: 59812-003
* Phone Number: 406-243-5769
Fax Number:
* E-Mail: molly.kimmel@mso.umt.edu

Credential, e.g., agency login:

*** Project Role:** Co-PD/PI
Other Project Role Category:

Degree Type: Master of Science
Degree Year: 2010

Attach Biographical Sketch	1244-Molly_Kimmel Biosketch.p	Add Attachment	Delete Attachment	View Attachment
Attach Current & Pending Support		Add Attachment	Delete Attachment	View Attachment

RESEARCH & RELATED Senior/Key Person Profile (Expanded)

PROFILE - Senior/Key Person 2

Prefix: | * First Name: Julia | Middle Name:
* Last Name: Mazzarella | Suffix:
Position/Title: Assistant Professor
Department: Office of Sponsored Programs
Organization Name: University of Montana
Division: Rural Institute
* Street1: 32 campus Dr. #2304
Street2:
* City: Missoula | County/ Parish: MT
* State: MT: Montana | Province:
* Country: USA: UNITED STATES | * Zip / Postal Code: 59812-003
* Phone Number: 406-243-4684 | Fax Number:
* E-Mail: julia.mazzarella@mso.umt.edu

Credential, e.g., agency login:

*** Project Role:** Co-PD/PI | **Other Project Role Category:**

Degree Type: PhD
Degree Year: 2023

Attach Biographical Sketch	1245-Julia_Mazzarella Biosket	Add Attachment	Delete Attachment	View Attachment
Attach Current & Pending Support		Add Attachment	Delete Attachment	View Attachment

PROFILE - Senior/Key Person 3

Prefix: | * First Name: Jennifer | Middle Name:
* Last Name: Banna | Suffix:
Position/Title: Family to Family Center Coordinator
Department: Office of Sponsored Programs
Organization Name: University of Montana
Division: Rural Institute
* Street1: 32 Campus Drive #2304
Street2:
* City: Missoula | County/ Parish:
* State: MT: Montana | Province:
* Country: USA: UNITED STATES | * Zip / Postal Code: 59812-0003
* Phone Number: 406-243-4570 | Fax Number:
* E-Mail: jennifer.banna@mso.umt.edu

Credential, e.g., agency login:

*** Project Role:** Faculty | **Other Project Role Category:**

Degree Type: Bachelor of Science
Degree Year: 1993

Attach Biographical Sketch	1246-Jenn_Banna Biosketch.pdf	Add Attachment	Delete Attachment	View Attachment
Attach Current & Pending Support		Add Attachment	Delete Attachment	View Attachment

OMB No. 0925-0001 and 0925-0002 (Rev. 10/2021 Approved Through 01/31/2026)

BIOGRAPHICAL SKETCH

NAME: McKearnan, Kimberly Ann

eRA COMMONS USER NAME (credential, e.g., agency login):

POSITION TITLE: Program Director and Assistant Professor, Occupational Therapy

EDUCATION/TRAINING

INSTITUTION AND LOCATION	DEGREE	Completion Date	FIELD OF STUDY
University of Puget Sound	BS	12/1992	Occupational Therapy
University of Washington	MS	08/1997	Physical Medicine and Rehabilitation (OT/PT combined program)
University of Washington	PhD	06/2004	Nursing Science

A. Personal Statement

I am an Assistant Professor and founding Program Director for the entry-level Occupational Therapy Doctorate (OTD) program. I have a broad background in clinical care, hospital & organizational leadership and academia. The majority of my career has been spent in pediatrics including acute care, outpatient, PICU/NICU, specialty care and, briefly, in school-based services. I have a long history of successful leadership of projects and teams including program development, strategic planning, and cultivating high-functioning teams. Given my diverse educational background and professional experiences, I have expertise in and am committed to promoting interprofessional collaboration in clinical care, research and education. As former faculty in the LEND program at Oregon Health and Sciences University, I am excited about the opportunity to bring this rich training program to the University of Montana to help address the gaps in care, training, and support across Montana.

B. Positions and Scientific Appointments

2023-present	Founding Program Director and Assistant Professor, Occupational Therapy Program (entry-level OTD), School of Speech, Language, Hearing and Occupational Sciences, College of Health, University of Montana, Missoula, MT
2023-present	Member, Advocacy Lead on the AOTA DD SIS Standing Committee
2016-2023	Executive Director/CEO- Children's Oncology Camp Foundation (d.b.a. Camp Mak-A-Dream), Missoula, MT
2015-2016	Division Administrator- Pediatric Hematology/Oncology, Neonatology, Developmental Pediatrics and Rehabilitation, Sidra Medical & Research Center, Doha, Qatar
2011-2015	Adjunct Faculty- University of Montana, Physical Therapy Department (Pediatric & Lifespan courses)
2008-2015	Director of Children's Services (Med-Surg Pediatrics (including Sedation Service, Outpatient Oncology, & Infusion Service), Pediatric ICU, Pediatric Specialty Clinics, & Neonatal ICU, Community Medical Center, Missoula, MT
2004-2008	Assistant Professor, Department of Pediatrics/Director of Occupational Therapy Services – Acute care, NICU, & pediatric specialty clinics; LEND program faculty; Oregon Health & Science University, Portland, OR
2002-2004	Pre-doctoral Research Associate, Management of Pain in Rehabilitation Medicine Project- Survey of Chronic Pain and It's Effects on Youth with Disabilities and The Efficacy of Relaxation Training for Youth with Disabilities (NIH funded), University of Washington, Seattle, WA
1992-2003	Lead inpatient therapist, Staff occupational therapist, traveling occupational therapist, various locations and settings

OMB No. 0925-0001 and 0925-0002 (Rev. 10/2021 Approved Through 01/31/2026)

BIOGRAPHICAL SKETCH

Provide the following information for the Senior/key personnel and other significant contributors.
Follow this format for each person. **DO NOT EXCEED FIVE PAGES.**

NAME: Molly Kimmel

eRA COMMONS USER NAME (credential, e.g., agency login):

POSITION TITLE: Director, Rural Institute for Inclusive Communities

EDUCATION/TRAINING *(Begin with baccalaureate or other initial professional education, such as nursing, include postdoctoral training and residency training if applicable. Add/delete rows as necessary.)*

INSTITUTION AND LOCATION	DEGREE *(if applicable)*	Completion Date MM/YYYY	FIELD OF STUDY
Gonzaga University	BA	05/2002	Psychology
University of Washington	MS	12/2010	Occupational Therapy

A. Personal Statement

As the Director of the Rural Institute, a University Center for Excellence in Developmental and related Disabilities, I oversee and provide fiscal management to 50+ projects and 60+ staff. I use community-driven leadership to guide the Rural Institute's education, research, and service initiatives and am the Principal Investigator on 6 projects totaling over $2 million in funding annually. I am a former LEND trainee and served as the Occupational Therapy Core Faculty member for URLEND between 2018 and 2024. During that time, I chaired the evaluation committee, contributed to the curriculum, and mentored all Montana-based trainees. With clinical training and experience as an occupational therapist, I have significant interdisciplinary service experience to go along with strong grant management and administrative skills.

B. Positions, Scientific Appointments, and Honors

- Fulbright Specialist in Indonesia (October 2025)
- Director of the Rural Institute for Inclusive Communities (June 2024 – Present)
- Interim Director of the Rural Institute (April 2023 – June 2024)
- Association of Assistive Technology Act Programs Board of Directors Member (February 2023 – present)
- Montana Occupational Therapy Association President (April 2021 – April 2025)
- Rural Dynamics Board of Directors Member (2020 - 2021 and Chair, 2022 – present)
- Statewide Independent Living Council Member (2020 – 2023)
- Program Director, MonTECH (April 2020 – Present)
- URLEND Core Faculty (May 2018 – May 2024)
- URLEND Trainee (August 2017 – April 2028)
- Occupational Therapist, St. Patrick Hospital (January 2011 – April 2022)

BIOGRAPHICAL SKETCH

NAME: Julia Mazzarella

eRA COMMONS USER NAME (credential, e.g., agency login): J-MAZZARELLA

ORCID: 0000-0001-9997-024X

POSITION TITLE: Assistant Professor

EDUCATION/TRAINING:

INSTITUTION AND LOCATION	DEGREE *(if applicable)*	Completion Date MM/YYYY	FIELD OF STUDY
Skidmore College, Saratoga Springs, NY	BA	05/2013	Neuroscience
Skidmore College, Saratoga Springs, NY	BA	05/2013	Psychology
The Ohio State University, Columbus, OH	DPT	05/2018	Physical Therapy
The Ohio State University, Columbus, OH	PhD	08/2022	Health and Rehabilitation Science
The Ohio State University, Columbus, OH	Post-doctoral Fellowship	06/2023	Clinical Trials, Biomechanics, Human-Animal Interaction

A. Personal Statement

I will serve as co-Program Director (PD) and Physical Therapy (PT) faculty for Montana LEND. I am well-suited to this role, as an Assistant Professor of PT with a focus on Pediatrics, and prior completion of 2 years as a LEND trainee with The Ohio State University's Nisonger Center. Additionally, my PhD included clinical and research training in pediatric physical therapy, with a focus on children with autism and neuromotor disorders. Morover, the aims of my research agenda align with LEND initiatives in striving to improve rehabilitation outcomes, life participation, and overall quality of life for children with autism and DD.

In my current role, I teach pediatrics to doctoral PT students, and I maintain a substantial research agenda. Thus, I am skilled in mentorship in educational, research, and clinical settings. In my second year as a LEND trainee, I fulfilled an *Advanced Trainee* role, which included teaching assistantship and mentorship of new trainees. With my prior experiences in LEND and training as a pediatric PT, researcher, and educator, I am exceptionally well-suited to fill both the PT faculty role and co-PD role for Montana LEND.

B. Positions, Scientific Appointments, and Honors

2023-present Assistant Professor, School of Physical Therapy and Rehabilitation Science, College of Health, University of Montana, Missoula, MT
2021-2023 Early Intervention Physical Therapist, The Childhood League Center, Columbus, OH
2019-2020 Albert Schweitzer Fellow, Columbus-Athens Schweitzer Fellows Program, Columbus, OH
2019-2020 Trainee Liaison, Association of University Centers on Disability (AUCD), Silver Spring, MD
2018-2022 Physical Therapist, The Ohio State University Hospital East, Columbus, OH
2018-2020 Trainee, Leadership Education in Neurodevelopmental Disabilities (LEND), OSU, Columbus, OH
2013-2014 Fulbright Student Researcher, Justus-Liebig Universität, Gießen, Hessen, Germany

C. Other Support

P20GM103474. *NIH*. 5/1/2025-4/30/2027. Role: PI
Title: PT for Montana Children with Neuromotor Disorders Using Hippotherapy and the Equine Environment

Major Research Project. *Horses and Humans Research Foundation (HHRF)*. 1/28/2026 – 1/27/2028. Role: PI
Title: Harnessing the Healing Power of Horses: An Interprofessional Approach to Post-Stroke Aphasia and Motor Recovery

R21 PA-25-306. *NICHD*. 6/9/2026-3/31/2028. Role: Site PI
Title: Tele-coaching for caregivers of young children with motor delays in rural communities

D. Other Information

I am fluent in the English language and have intermediate proficiency in German. I also have prior experience providing PT services in Brazil with a patient population culturally and linguistically different than my own.

OMB No. 0925-0001 and 0925-0002 (Rev. 10/2021 Approved Through 01/31/2026)

BIOGRAPHICAL SKETCH

NAME: Jennifer Banna

eRA COMMONS USER NAME (credential, e.g., agency login):

POSITION TITLE: Family to Family Center Coordinator

EDUCATION/TRAINING

INSTITUTION AND LOCATION	DEGREE	Completion Date	FIELD OF STUDY
Montana State University	BS	12/1993	Home Economics/Child Development/Family Science
Montana State University	BS	12/1993	Elementary Education

A. Personal Statement

Jennifer Banna brings many years of experience supporting families of children and youth with special health care needs, grounded in both professional work and lived experience as a parent. As Center Coordinator for the Montana Family to Family Health Information Center, she works directly with families and providers to help them find resources, understand their choices, and navigate systems like Medicaid, Title V, and developmental disability services. She is known for taking complex systems and making them understandable and usable for families. A big part of her work is building relationships across the state with agencies, providers, and community partners, and helping connect those efforts so families can access what they need. She demonstrates strong leadership in family engagement and systems of care, making sure family perspectives are included from the start and throughout projects. She recently completed the Family Engagement and Leadership in Systems of Care (FELSC) program through Family Voices, which strengthened her ability to work across systems and elevate family voice. Jennifer leads and supports the AMCHP Family Delegate role, helping ensure meaningful family engagement in state and national maternal and child health work. She also oversees plain language contracts to make complex information easier for families and self-advocates to understand and use.

B. Positions

Parent, Nana (caregiver and advocate), Missoula, Montana; 1994 – Present

Utah Regional Leadership Education in Neurodevelopmental and Related Disabilities (URLEND) Family Trainee; Salt Lake City, Utah; Completed: May 2014

Center Coordinator, Montana Family to Family Health Information Center
Rural Institute for Inclusive Communities, University of Montana; June 2019 – Present

Association of Maternal and Child Health Programs Leadership Lab
Family Leaders Cohort
Completed: 2022

Project/Performance Site Location(s)

Project/Performance Site Primary Location

☐ I am submitting an application as an individual, and not on behalf of a company, state, local or tribal government, academia, or other type of organization.

Organization Name: University of Montana

UEI: DAY7Z8ZD48Q3

* Street1: 32 Campus Drive

Street2:

* City: Missoula

County:

* State: MT: Montana

Province:

* Country: USA: UNITED STATES

* ZIP / Postal Code: 59812-0003

* Project/ Performance Site Congressional District: MT-001

Project/Performance Site Location 1

☐ I am submitting an application as an individual, and not on behalf of a company, state, local or tribal government, academia, or other type of organization.

Organization Name:

UEI:

* Street1:

Street2:

* City:

County:

* State:

Province:

* Country: USA: UNITED STATES

* ZIP / Postal Code:

* Project/ Performance Site Congressional District:

Additional Location(s)

Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 1

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: DAY7Z8ZD48Q3 **Enter name of Organization:** University of Montana

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 1 **Start Date:** 09/30/2026 **End Date:** 06/30/2027

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Kimberly	Ann	McKearnan		94,089.78		3.51	1.29	51,497.00	17,326.00	68,823.00
Project Role: PD/PI											
	Molly		Kimmel		159,688.10	3.00			39,922.00	15,912.00	55,834.00
Project Role: Co-PI											
	Julia		Mazzarella		87,161.49		1.75	0.65	23,852.00	8,189.00	32,041.00
Project Role: Co-PD											
	Jennifer		Banna		54,175.01		1.20		5,418.00	3,932.00	9,350.00
Project Role: Family/Self Advocate											

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 166,048.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
9	Faculty Leads/Core Faculty/Affiliate Faculty	15.24	0.44	0.32	91,745.00	45,966.00	137,711.00
9	**Total Number Other Personnel**					**Total Other Personnel**	137,711.00

Total Salary, Wages and Fringe Benefits (A+B) 303,759.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file	
Total Equipment	

D. Travel

	Funds Requested ($)
1. Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	25,193.00
2. Foreign Travel Costs	
Total Travel Cost	25,193.00

E. Participant/Trainee Support Costs

	Funds Requested ($)
1. Tuition/Fees/Health Insurance	
2. Stipends	58,500.00
3. Travel	3,000.00
4. Subsistence	
5. Other	
Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	61,500.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	10,150.00
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	28,210.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Communications/Postage	1,000.00
9.	Meetings/Conference room rental	500.00
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	39,860.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	430,312.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
Training Grant Rate; Modified Total Direct Costs (MTDC) Base	8.00	368,812.00	29,505.00
		Total Indirect Costs	29,505.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)

Department of Health and Human Services
Jeanette Lu
(415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	459,817.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	459,817.00

L. Budget Justification

(Only attach one file.) 1234-Budget-Justification_McKearnan.pd Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 2

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: DAY7Z8ZD48Q3 **Enter name of Organization:** University of Montana

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 2 **Start Date:** 07/01/2027 **End Date:** 06/30/2028

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Kimberly	Ann	McKearnan		97,382.92		3.29	0.97	47,281.00	16,119.00	63,400.00
Project Role:	PD/PI										
	Molly		Kimmel		165,277.20	3.00			41,319.00	16,452.00	57,771.00
Project Role:	Co-PI										
	Julia		Mazzarella		90,212.14		1.75	0.65	24,687.00	8,465.00	33,152.00
Project Role:	Co-PD										
	Jennifer		Banna		56,071.13	1.20			5,607.00	4,058.00	9,665.00
Project Role:	Family/Self Advocate Faculty										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 163,988.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
9	Faculty Leads/Core Faculty/Affiliate Faculty	14.40	0.44	0.32	89,070.00	44,497.00	133,567.00
9	**Total Number Other Personnel**					**Total Other Personnel**	133,567.00

Total Salary, Wages and Fringe Benefits (A+B) 297,555.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	27,780.00
2.	Foreign Travel Costs	
	Total Travel Cost	27,780.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	58,500.00
3.	Travel	2,500.00
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	61,000.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	11,150.00
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	31,210.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Communications/Postage	1,000.00
9.	Meetings/Conference room rental	300.00
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	43,660.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	429,995.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
Training Grant Rate; Modified Total Direct Costs (MTDC) Base	8.00	368,995.00	29,520.00
		Total Indirect Costs	29,520.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)

Department of Health and Human Services
Jeanette Lu
(415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	459,515.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	459,515.00

L. Budget Justification

(Only attach one file.) 1234-Budget-Justification_McKearnan.pd Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 3

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: DAY7Z8ZD48Q3 **Enter name of Organization:** University of Montana

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 3 **Start Date:** 07/01/2028 **End Date:** 06/30/2029

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Kimberly	Ann	McKearnan		100,791.30		2.63	0.97	41,373.00	13,885.00	55,258.00
Project Role: PD/PI											
	Molly		Kimmel		171,061.90	1.80			30,791.00	12,248.00	43,039.00
Project Role: Co-PI											
	Julia		Mazzarella		93,369.57		1.75	0.65	25,552.00	8,750.00	34,302.00
Project Role: Co-PD											
	Jennifer		Banna		58,033.62	1.20			5,803.00	4,188.00	9,991.00
Project Role: Family/Self Advocate Faculty											

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file:

Total Senior/Key Person: 142,590.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
9	Faculty Leads/Core Faculty/Affiliate Faculty	14.64	0.44	0.32	85,157.00	42,619.00	127,776.00
9	**Total Number Other Personnel**					**Total Other Personnel**	127,776.00

Total Salary, Wages and Fringe Benefits (A+B) 270,366.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	23,439.00
2.	Foreign Travel Costs	
	Total Travel Cost	23,439.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	103,500.00
3.	Travel	1,000.00
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	104,500.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	3,150.00
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	31,210.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Communications/Postage	400.00
9.	Meetings/Conference room rental	300.00
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	35,060.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	433,365.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
Training Grant Rate; Modified Total Direct Costs (MTDC) Base	8.00	328,865.00	26,309.00
		Total Indirect Costs	26,309.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)

Department of Health and Human Services
Jeanette Lu
(415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	459,674.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	459,674.00

L. Budget Justification

(Only attach one file.) 1234-Budget-Justification_McKearnan.pd Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 4

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: DAY7Z8ZD48Q3 **Enter name of Organization:** University of Montana

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 4 **Start Date:** 07/01/2029 **End Date:** 06/30/2030

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Kimberly	Ann	McKearnan		104,319.02		2.63	0.97	42,822.00	14,354.00	57,176.00
Project Role: PD/PI											
	Molly		Kimmel		177,049.03	1.80			26,557.00	10,553.00	37,110.00
Project Role: Co-PI											
	Julia		Mazzarella		96,637.50		1.58	0.65	24,517.00	8,319.00	32,836.00
Project Role: Co-PD											
	Jennifer		Banna		60,064.79	1.08			5,406.00	3,891.00	9,297.00
Project Role: Family/Self Advocate Faculty											

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 136,419.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
9	Faculty Leads/Core Faculty/Affiliate Faculty	13.44	0.44	0.32	88,139.00	44,026.00	132,165.00
9	**Total Number Other Personnel**					**Total Other Personnel**	132,165.00

Total Salary, Wages and Fringe Benefits (A+B) 268,584.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	25,663.00
2.	Foreign Travel Costs	
	Total Travel Cost	25,663.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	103,500.00
3.	Travel	1,000.00
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	104,500.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	1,650.00
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	32,210.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Communications/Postage	400.00
9.	Meetings/Conference room rental	300.00
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	34,560.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	433,307.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
Training Grant Rate; Modified Total Direct Costs (MTDC) Base	8.00	328,807.00	26,305.00
		Total Indirect Costs	26,305.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)

Department of Health and Human Services
Jeanette Lu
(415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	459,612.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	459,612.00

L. Budget Justification

(Only attach one file.) 1234-Budget-Justification_McKearnan.pd Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 5

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: DAY7Z8ZD48Q3 **Enter name of Organization:** University of Montana

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 5 **Start Date:** 07/01/2030 **End Date:** 06/30/2031

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Kimberly	Ann	McKearnan		107,970.19		2.63	0.97	44,321.00	14,839.00	59,160.00
Project Role:	PD/PI										
	Molly		Kimmel		183,245.74	1.80			27,487.00	10,912.00	38,399.00
Project Role:	Co-PI										
	Julia		Mazzarella		100,019.81		1.58	0.65	25,375.00	8,599.00	33,974.00
Project Role:	Co-PD										
	Jennifer		Banna		62,167.06	0.96			4,973.00	3,569.00	8,542.00
Project Role:	Family/Self Advocate Faculty										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 140,075.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
9	Faculty Leads/Core Faculty/Affiliate Faculty	13.08	0.44	0.32	88,582.00	44,197.00	132,779.00
9	**Total Number Other Personnel**					**Total Other Personnel**	132,779.00

Total Salary, Wages and Fringe Benefits (A+B) 272,854.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file	
Total Equipment	

D. Travel

	Funds Requested ($)
1. Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	23,674.00
2. Foreign Travel Costs	
Total Travel Cost	23,674.00

E. Participant/Trainee Support Costs

	Funds Requested ($)
1. Tuition/Fees/Health Insurance	
2. Stipends	103,500.00
3. Travel	200.00
4. Subsistence	
5. Other	
Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	103,700.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	900.00
2.	Publication Costs	
3.	Consultant Services	
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	32,210.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Communications/Postage	200.00
9.	Meetings/Conference room rental	50.00
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	33,360.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	433,588.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
Training Grant Rate; Modified Total Direct Costs (MTDC) Base	8.00	329,888.00	26,391.00
		Total Indirect Costs	26,391.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)

Department of Health and Human Services
Jeanette Lu
(415) 437-7820

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	459,979.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	459,979.00

L. Budget Justification

(Only attach one file.) 1234-Budget-Justification_McKearnan.pd Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Cumulative Budget

		Totals ($)
Section A, Senior/Key Person		749,120.00
Section B, Other Personnel		663,998.00
Total Number Other Personnel	45	
Total Salary, Wages and Fringe Benefits (A+B)		1,413,118.00
Section C, Equipment		
Section D, Travel		125,749.00
1. Domestic	125,749.00	
2. Foreign		
Section E, Participant/Trainee Support Costs		435,200.00
1. Tuition/Fees/Health Insurance		
2. Stipends	427,500.00	
3. Travel	7,700.00	
4. Subsistence		
5. Other		
6. Number of Participants/Trainees		
Section F, Other Direct Costs		186,500.00
1. Materials and Supplies	27,000.00	
2. Publication Costs		
3. Consultant Services		
4. ADP/Computer Services		
5. Subawards/Consortium/Contractual Costs	155,050.00	
6. Equipment or Facility Rental/User Fees		
7. Alterations and Renovations		
8. Other 1	3,000.00	
9. Other 2	1,450.00	
10. Other 3		
11. Other 4		
12. Other 5		
13. Other 6		
14. Other 7		
15. Other 8		
16. Other 9		
17. Other 10		

Section G, Direct Costs (A thru F)	2,160,567.00
Section H, Indirect Costs	138,030.00
Section I, Total Direct and Indirect Costs (G + H)	2,298,597.00
Section J, Fee	
Section K, Total Costs and Fee (I + J)	2,298,597.00

RESEARCH & RELATED Other Project Information

OMB Number: 4040-0001
Expiration Date: 01/31/2029

1. Are Human Subjects Involved? ☐ Yes ☒ No

1.a. If YES to Human Subjects

Is the Project Exempt from Federal regulations? ☐ Yes ☐ No

If yes, check appropriate exemption number. ☐ 1 ☐ 2 ☐ 3 ☐ 4 ☐ 5 ☐ 6 ☐ 7 ☐ 8

If no, is the IRB review Pending? ☐ Yes ☐ No

IRB Approval Date:

Human Subject Assurance Number:

2. Are Vertebrate Animals Used? ☐ Yes ☒ No

2.a. If YES to Vertebrate Animals

Is the IACUC review Pending? ☐ Yes ☐ No

IACUC Approval Date:

Animal Welfare Assurance Number:

3. Is proprietary/privileged information included in the application? ☐ Yes ☒ No

4.a. Does this Project Have an Actual or Potential Impact - positive or negative - on the environment? ☐ Yes ☒ No

4.b. If yes, please explain:

4.c. If this project has an actual or potential impact on the environment, has an exemption been authorized or an environmental assessment (EA) or environmental impact statement (EIS) been performed? ☐ Yes ☐ No

4.d. If yes, please explain:

5. Is the research performance site designated, or eligible to be designated, as a historic place? ☐ Yes ☒ No

5.a. If yes, please explain:

6. Does this project involve activities outside of the United States or partnerships with international collaborators? ☐ Yes ☒ No

6.a. If yes, identify countries:

6.b. Optional Explanation:

7. Project Summary/Abstract 1235-Project-Abstract.pdf Add Attachment Delete Attachment View Attachment

8. Project Narrative 1236-MT LEND Project Narrative.pdf Add Attachment Delete Attachment View Attachment

9. Bibliography & References Cited 1237-References Cited.pdf Add Attachment Delete Attachment View Attachment

10. Facilities & Other Resources Add Attachment Delete Attachment View Attachment

11. Equipment Add Attachment Delete Attachment View Attachment

12. Other Attachments Add Attachments Delete Attachments View Attachments

Introduction

The Montana Leadership Education in Neurodevelopmental and Other Related Disabilities Program (Montana LEND) would be initiated in Fall 2026 as the **first standalone LEND program based in the state of Montana**. This would be the culmination of years of documented need alongside interest in and dedication to establishing a program in Montana that would align with the unique attributes, challenges, and opportunities in our rural, frontier state.

We will partner with our established University Center for Excellence in Developmental and related Disabilities (UCEDD), the Rural Institute for Inclusive Communities. Located on the University of Montana (UM) campus, we will enhance and expand their established and productive track record of mentoring, training, continuing education, service delivery, and research.

The mission of Montana LEND is **to improve the health-related quality of life of Montanans with neurodevelopmental and related disabilities**. We will train interdisciplinary leaders who work together to provide family-centered and culturally relevant early detection and care across the lifespan while advocating for systems-level change. Through collaboration, innovation, and intentional engagement with Montana's rural and tribal communities, the program will advance fully integrated care for children, youth and young adults with autism spectrum disorder (ASD) and other developmental disabilities. Collectively, this will improve access and outcomes for individuals and families across the Big Sky state.

As a newly funded program, our plan is to strategically **build and expand training, services, continuing education (CE), and technical assistance (TA)** tailored to learners, families, providers, and communities in our state. We will recruit for a minimum of 9 long-term trainees in at least 7 disciplines, including an individual with a disability and a family trainee with lived experience of disability. We will provide a **robust, 2-semester (academic year) interdisciplinary curriculum including didactic, clinical, and leadership/research elements**. By Year 3, we intend to incorporate a clinical fellow who, in addition to the standard expectations of LEND program trainees, will be trained in more advanced clinical evaluation, diagnostic, and intervention skills that will enhance our program's ability to increase the expertise and capacity of services in Montana. Our core faculty upon initiation will be comprised of at least 9 disciplines, including individuals with disabilities and family faculty members.

We will initially focus on an in-person format; However, by Year 2, we will have a virtual option for trainees who live too far from Missoula to travel here regularly. By Year 3, we will transition to offering a **fully hybrid** program to ensure we are able to include trainees from all corners of our large state. We will facilitate diverse, evidence-based didactic training, skill- and competency-building leadership and/or research experiences, and rich clinical opportunities under the mentorship of experienced faculty and in response to identified community needs. Additionally, by Year 3, Montana LEND faculty plan to utilize strategic partnerships and leveraged funding to open a **new interdisciplinary pediatric therapy clinic at the University of Montana**. The clinic will provide trainees with hands-on experience in interdisciplinary practice, ensuring they complete the program with the knowledge and skills needed to collaborate

effectively and deliver coordinated, family-centered services to Montana children and families.

In collaboration with our partners and utilizing the expertise of our faculty, we will also provide evidence-based CE or professional development opportunities and TA to health professionals, organizations, agencies and individuals in communities across our state.

To assist Montana LEND in effectively and efficiently meeting their goals, a **well-established evaluation team** will work alongside Montana LEND faculty and trainees providing iterative program evaluation services. Together, they will ensure Montana LEND provides quality experiences and instruction to all trainees, while also meeting the needs of the disability-serving workforce.

Need

Nationally, **children and youth with special health care needs (CYSHCN) represent a substantial and growing population**, with approximately 21% of children in the United States or more than 15 million, affected.[1] Within this group, **ASD prevalence has increased significantly**, from 1 in 150 children in 2000 to approximately 1 in 25 (3.9%) in 2023,[2,3] representing more than 2 million children. ASD occurs across all population groups and occurs in boys more than three times the rate of girls.[4]

This increasing prevalence places growing demands on systems of care that are already strained. Nationally, 82% of children with special health care needs do not receive care within a well-functioning system,[5] 54% lack access to a medical home,[6] and 90% are not receiving needed therapies.[7] Nearly one-quarter of children experience difficulty obtaining referrals.[8] These gaps are driven in part by provider shortages[9,10,11] and **practitioners who lack the knowledge and training to provide quality care to individuals with ASD and intellectual disability**.[12] This underscores the need for a well-prepared, interdisciplinary workforce, especially in rural and remote areas with fewer established resources.

Montana Context

In Montana, geography and demographics shape healthcare access for CYSHCN. As the fourth largest state by land area (145,550 square miles) with a relatively small population of 1,084,225,[13] Montana is characterized by vast distances between communities, and approximately 33% of residents live in rural or frontier areas.[14] American Indians represent the state's largest minority population at 6.2%,[14] with 12 federally recognized tribes and 7 reservations distributed across the state.[15] Taken together, **geographic isolation, limited infrastructure, and the presence of sovereign tribal nations with distinct health systems and needs create compounding barriers to accessing specialized, coordinated care** for children with autism and other developmental disabilities.

In Montana, approximately 3.2% of children have a diagnosis of ASD,[16] reflecting the national trend of rising prevalence. Yet the state's service infrastructure struggles to meet this need. **Seventy-eight percent of Montana CYSHCN do not receive services in a well-functioning healthcare system,[17] and nearly half (49.7%) lack access to a medical home approach to care.**[18] These gaps have real consequences for families seeking care, with 20.6% of Montana children reporting at least some difficulty obtaining a referral to needed services.[19]

To better understand how these gaps affect families and providers across Montana's urban and rural communities, we conducted listening sessions, interviews, focus groups, and surveys with results as follows.

Montana Advocacy Organizations

We conducted listening sessions with the Developmental Disabilities Council (DD Council) and the Family Support Services Advisory Council (FSSAC), a division of the Montana Department of Public Health and Human Services (DPHHS), which produced several themes. Group members identified **limited access to services and resources, workforce shortages, and unmet needs for children with co-occurring developmental disabilities and behavioral health conditions** as pressing concerns. They also highlighted **gaps in provider training** as a barrier to the delivery of evidence-based practices. Of note were the **lack of in-state specialty services**, which forces many families to travel long distances to access care, and lengthy waitlists for diagnostic evaluations. Additionally, they conveyed the **need for transition support from pediatric to adult services**, describing a "cliff" experienced by individuals with complex needs as they age out of pediatric care.

Regarding provider knowledge and practice, participants emphasized the **need for increased interprofessional collaboration** to better address the needs of the whole individual. Participants highlighted the importance of **equipping professionals with the counseling skills needed to empower caregivers to effectively advocate** for themselves and their families. Lastly, they also underscored the **importance of provider awareness related to the lasting impact of both spoken and written communication**. Words shared during appointments often extend far beyond the clinical encounter, and documentation within medical records can shape how children and families are perceived and supported over time. These communications can influence not only the trajectory of medical care but also families' emotional well-being and overall experience navigating the healthcare system.

Maternal Child Health State Agencies

Interviews with the Montana Autism Education Project, the Montana Developmental Disabilities Program, and Children's Special Health Services (the latter two of which are housed within Montana DPHHS) reinforced several of the themes identified by advocacy organizations. Agency representatives identified two of the most pressing needs for Montana children and families as **improved access to timely, high-quality differential diagnostic services delivered by well-trained providers, and increased in-state services for children with co-occurring developmental and behavioral health diagnoses**. Emerging priorities included expanded transition and post-secondary pathways for youth with autism and other developmental disabilities and addressing rural health disparities through more equitable access to services across Montana's varied landscape. When asked about training and TA needs, agency representatives stated priorities as provider training in the Autism Diagnostic Observation Schedule, 2nd edition (ADOS-2) and Autism Diagnostic Interview-Revised (ADI-R) to **strengthen differential diagnostic capabilities, behavioral and intervention training for direct service providers, and specialty training for pediatricians and other providers in rural areas** to deepen their understanding of ASD and related neurodevelopmental disabilities.

A separate needs assessment of Montana's Title V Maternal and Child Health Block Grant was published by DPHHS in August 2025.[20] Many of the findings align with themes in this report. Key barriers shared by maternal and child health professionals from DPHHS and nonprofits were **funding and workforce shortages, access and infrastructure challenges in a rural state, and stigma around mental health**.

Families, Caregivers, and Young Adults with Disabilities

A survey distributed to family members, caregivers of CYSHCN, and young adults with disabilities across Montana yielded 53 responses, offering insight into the experiences of those most affected by the state's service gaps. Respondents identified their most pressing needs as **coordinated health services, help paying for services, access to in-person or telehealth medical and mental health care, and access to providers with specialized training**. Families also identified information and navigation support as critical unmet needs, including more information on available resources, help understanding insurance coverage, and access to support groups and community resources. One parent's experience illustrates the gap in coordinated care: *"My healthcare provider sent us to a specialist but not to the resources in our community that would help pay for the services and connect us with a case manager."* This feedback underscores the **need for a stronger medical home model in Montana,** where providers are equipped to support families beyond the clinical encounter and are knowledgeable about rural health systems, community resources, and the navigation support that families need to access care.

Further, wait times emerged as a significant barrier, with 67% of respondents identifying long appointment wait times as a problem when seeking care for their child. **Families reported waiting six months to a year for diagnostic evaluations and treatment across specialties.** As one parent noted, *"There is a need for more providers; waiting months for an appointment should not happen."* These experiences reflect the direct human cost of Montana's provider shortage and reinforce the need for a well-trained, interdisciplinary workforce capable of meeting demand across the state.

In DPHHS' recent Montana Title V needs assessment, families shared they need **support paying for healthcare (40%) and access to mental health services (34%)**. One parent was quoted saying their family lives 70 minutes from the nearest hospital; a worrisome situation if an emergency medical event occurs.[20] These themes are consistent with what was shared by families in this needs assessment: **healthcare has an affordability crisis, and access to services is a barrier for CYSHCN.**

Montana Healthcare Providers

To capture the perspectives of Montana's direct service workforce, we surveyed 63 healthcare providers representing a range of disciplines and practice settings. Respondents included occupational therapists or certified occupational therapy assistants (OT/COTA, 24%), speech-language pathologists (SLP, 22%), nurses or nurse practitioners (17%), physicians (10%), physical therapists or physical therapy assistants (PT/PTA, 8%), social workers (3%), and other professionals including clinic directors, special education teachers, and special education directors (16%). Providers practiced across hospital (29%), outpatient clinic (24%), school or university (21%), private practice (17%), primary care (2%), and other settings (8%). Fifty-four percent of

the respondents indicated they live and work in an urban setting while 46% identified as being rural.

When given a list of providers modeled after LEND trainee types, providers were asked to check which professionals are most needed in Montana (no limit on responses). Respondents identified OTs and COTAs (66%), mental health counselors (59%), social workers (49%), SLPs (46%), psychologists (46%), PTs and PTAs (39%), physicians (36%), dietitians (33%), genetic counselors (31%), and audiologists (25%) as priority workforce needs.

An open-ended question asked providers to identify any significant barriers to serving children with special health care needs. **The most frequently cited barriers were care coordination challenges (71%), lack of access to specialty care (67%), provider shortages (57%), and long waitlists (43%).** These findings reinforce the systemic gaps identified by advocacy organizations and state agencies in the preceding sections.

Regarding training and education needs, providers pointed to three broad areas of priority: **clinical competency, interprofessional collaboration, and coordinated systems of care**. More specifically, respondents identified training in multidisciplinary assessment, diagnostic skills, and evidence-based interventions specific to ASD, as well as CE in pediatric special health care needs, family-centered care, and available community resources, as areas of greatest need.

Past LEND Trainees Representing Montana

Fifteen former trainees and fellows from LEND programs across the United States, who now live and work in Montana, responded to a survey about needs and emerging priorities. Responses reinforce many of the gaps identified across stakeholder groups, while offering insight from individuals with advanced interdisciplinary training and lived experience within service systems. Trainees identified **persistent barriers to accessing both primary and specialty care**, particularly in rural and frontier areas where geographic distance, limited provider availability, and unreliable internet connectivity constrain access to timely services. Long waitlists for diagnostic evaluations, therapy, and Medicaid waiver barriers further delay care, while families often lack adequate care coordination, case management, and guidance in navigating complex systems following diagnosis. Trainees also emphasized **gaps in provider knowledge**, including limited understanding of available resources, insufficient training in neurodevelopmental and related disabilities, and a need for more professionals prepared to deliver family-centered and holistic care.

Former LEND trainees identified strengthening workforce capacity and improving care coordination as critical priorities for Montana. This includes increasing the number of well-trained providers across disciplines, enhancing interdisciplinary collaboration, and expanding training in trauma-informed and family-centered practices. Trainees also highlighted the need for greater investment in family support, including caregiver education, peer mentoring, and healthcare system navigation assistance, as well as broader community-based training for professionals such as childcare providers and first responders. Concerns about the rising cost of care, affordability of insurance, and potential threats to Medicaid and disability funding further underscore the need for a workforce equipped to support families and advocate for sustainable systems of care. Together, these perspectives highlight the importance of preparing leaders who can

address both immediate service gaps and long-term system challenges impacting children and youth with autism and developmental disabilities in Montana.

Community Cultural Partners

In Montana, American Indian communities represent a critical voice in understanding the landscape of CYSHCN. Montana's population includes 6.2% individuals who identify as American Indian;[14] when including those who identify as multiracial, this percentage becomes much larger. Montana is home to 12 federally recognized Tribal Nations. Tribal members live within 7 reservation communities, as well as rural and urban communities across the state. Meaningful partnership with these communities are essential to building a LEND program that is responsive to the needs of the indigenous people. **A focus group was conducted with members of the Montana Native American Maternal Health Community Advisory Board, with attendees representing the Blackfeet, Rocky Boy, Flathead, Northern Cheyenne, Fort Peck Sioux, Bitterroot Salish, and Nakoda and Aaniiih Tribes of Fort Belknap.** Participants brought a range of perspectives, including those of academic nurses, birth workers, labor/delivery nurses, PTs, home visitors, policy and advocacy professionals, family services providers, and individuals with lived experience as parents.

Many of the needs identified by focus group participants echoed themes raised by other stakeholder groups, including shortages of healthcare professionals, inadequate referral pathways, and limited access to specialists such as SLPs, OTs/COTAs, and mental health providers. Participants also identified **access to transportation and peer and family support groups as pressing unmet needs.** The severity of provider shortages in tribal communities was captured by one participant: *"Our medical needs are being unmet. And it's scary, but it's a fact. And like I said, we only have one or two pediatricians to service over 5,000 tribal members that live here."*

Beyond provider availability, focus group participants identified a **lack of cultural humility among healthcare providers** as a significant barrier to care. As one participant observed, *"Some [healthcare practitioners] just are not taking the time to understand or to acknowledge that there are differences in culture, and there are cultural values that are important."* The same participant offered a clear path forward: *"If there's a way to make that connection back to culture, or even include culture in treatment and whatnot, then it'll become more relevant."* This feedback highlights the importance of training providers not only in clinical skills but in the cultural values, community contexts, and relational practices that make care meaningful and accessible for American Indian families.

Looking ahead, **focus group participants articulated a vision of care that is community-based and culturally grounded**. As one participant shared, *"[I hope] families can receive care at home. Maybe contracting a provider who would come into the community and give care to families in need."* This vision aligns directly with the LEND program's emphasis on community-based training and its commitment to preparing providers to deliver care across diverse settings and populations, including Montana's tribal communities.

Montana LEND Impact

Access to services emerged as a consistent and urgent theme across this needs assessment, particularly the shortage of providers across disciplines who are

adequately prepared to serve children and youth with autism and developmental disabilities within coordinated, family-centered systems of care. **Montana LEND will directly address these gaps by developing an interdisciplinary workforce equipped with the competencies necessary to improve access, quality, and coordination of care.** Grounded in the Maternal and Child Health (MCH) leadership framework and LEND core competencies, the program will provide yearlong, interdisciplinary training that emphasizes evidence-based practice, interprofessional collaboration, family/professional partnership, and systems-level leadership.

Montana LEND will prepare trainees across disciplines to function as leaders within complex and resource-limited systems. **Trainees will gain skills in care coordination, screening and diagnosis, and delivering culturally and linguistically appropriate services, while also developing the capacity to address systemic barriers such as fragmented referral pathways, limited provider availability, and gaps in rural service delivery.** For example, a LEND-trained provider practicing in a rural or frontier community will be equipped not only to deliver high-quality clinical care, but also to strengthen the local system of care by coordinating across sectors, connecting families to peer supports such as the Family-to-Family Health Information Center, and advancing a medical home model that integrates services across settings. In this way, Montana LEND directly responds to the above-described needs related to care coordination, family navigation, and access to timely services.

Montana LEND will also emphasize cultural humility and meaningful engagement with families and communities, including American Indian populations. **The Montana LEND curriculum will support trainees in understanding the cultural, geographic, and systemic contexts that shape access to care in Montana, and in integrating community knowledge and values into service delivery.** This focus is essential given stakeholder-identified gaps in culturally responsive care and evidence demonstrating that culturally sensitive care is associated with significantly improved child health outcomes.[21]

By increasing the number of providers trained in interdisciplinary, culturally responsive, and systems-oriented care, **Montana LEND will strengthen Montana's MCH workforce and improve the state's system of care for children and youth with ASD/DD**. Over time, this investment in workforce development will contribute to reduced wait times, improved access to diagnostic and treatment services, enhanced care coordination, and improved outcomes for children and families across Montana's rural, frontier, and tribal communities.

Approach

Overall Methodology

The Montana LEND core faculty comprises former LEND trainees and faculty from across the country. Over the past two and a half years, the core faculty has met regularly to review community input and thoughtfully develop a LEND training program **tailored to meet the unique needs of Montanans with autism and other developmental disabilities**.

With representation from multiple departments at UM, as well as a strong partnership with the Rural Institute for Inclusive Communities, the faculty is well positioned to recruit

qualified trainees from nearly all disciplines represented within the LEND framework in Montana (see *Attachments 2 and 4* for a staffing plan and project organizational chart.) This collaborative structure supports the delivery of a comprehensive skills training and educational experience. The faculty have already engaged in outreach with clinical and state Title V partners and maintain strong, established relationships with providers and community partners throughout Montana. They also bring extensive experience in delivering CE and TA to populations served by LEND programs.

Using additional grant funding and existing partnerships, **Montana LEND faculty will open an interdisciplinary care clinic on the campus of the University of Montana in Year 3** that will support short-, medium-, and long-term trainees, and clinical fellows will provide an integral piece of the training framework. The Montana LEND faculty is fully prepared and enthusiastic about launching and sustaining this program.

Goals and Objectives

Goal One: Establish and implement a LEND program in Montana that meets the unique needs and attributes of our state by creating the infrastructure, curriculum and partnerships to train and support postgraduate students, family members, individuals with disabilities, and practicing professionals dedicated to improving care for children and youth with autism and other developmental disabilities.

Objective 1.1 Implement the organizational infrastructure for a Montana LEND program, including faculty, facilities, partner and/or clinical experience agreements, virtual education or service platform, and administrative services.

Objective 1.2 Develop, conduct, and evaluate a 2-semester (academic year) community-driven interdisciplinary curriculum for the didactic, clinical, and leadership/research aspects of the Montana LEND program in collaboration with core faculty (including a self-advocate and family member), community partners, and related staff.

Objective 1.3 Recruit nine multi-disciplinary trainees yearly from across Montana, including one family member and one individual with autism or other developmental disability by Year 2.

Objective 1.4 By Year 3, implement a clinical fellow traineeship, for at least 1 trainee, that will be comprised of the standard Montana LEND curriculum in addition to further training and clinical experiences focused on working with children, youth and young adults with autism and developmental disabilities.

Goal Two: Provide education, training, technical assistance, and access to evidence-based research and resources to increase Montana provider capability and capacity to evaluate, diagnose and provide evidence-based interventions for children, youth and young adults with autism and other developmental disabilities.

Objective 2.1 Organize and deliver at least four continuing education and/or professional development opportunities yearly for healthcare and disability providers and community members across Montana, developed by Mondana LEND faculty, trainees, and/or clinical partners.

Objective 2.2 Montana LEND faculty, staff, and clinical partners will serve as a valued resource for technical assistance to at least fifteen disability-related organizations across Montana, including to rural and tribal communities.

Objective 2.3 At least three leadership projects will be developed yearly using community input and disseminated for use across the state upon completion.

Objective 2.4 In partnership with the University of Montana and Montana Department of Public Health and Human Services, use leveraged funding to open and operate an interdisciplinary evaluation, diagnostic, and treatment clinic on the University of Montana campus by Year 3.

Goal Three: Develop and strengthen partnerships across Montana, with emphasis in rural, tribal, and frontier communities, to increase access to coordinated, high-quality care and support services for children, youth, and young adults with autism and other developmental disabilities.

Objective 3.1 By Year 3, Montana LEND will establish and maintain a minimum of eight formal partnerships (MOUs or agreements) with providers/clinics, schools, tribal health organizations, Title V funded programs and community groups across rural, tribal, and frontier regions for meaningful collaboration with Montana LEND.

Objective 3.2 Host at least 4 community-based outreach events per year in a combination of rural, tribal, and urban communities, reaching at least 100 families and caregivers yearly.

Objective 3.3 Engage at least 10 short- and/or medium-term trainees from across Montana yearly who participate in didactic sessions eligible for continuing education credit for allied health professionals.

Trainee Recruitment and Retention

Montana LEND will be a Tier 1 program with 9 long-term trainees (LTTs) and 10 medium- or short-term trainees per year. The LTT cohort will include 1 each of 7 different disciplines, plus 1 family trainee, and 1 trainee with a disability. Montana LEND will recruit this interdisciplinary cohort of trainees from a variety of places, including students in the Montana University System (MUS) enrolled in relevant health professions programs, licensed professionals already working in the state, and family- and self-advocates living in the state. The MUS consists of 16 institutions across the state that graduate approximately 1000 health professions students annually.[22,23] **Specific LEND disciplines/programs offered through MUS include: PT, OT, SLP, public health, social work, clinical psychology, school psychology/counseling, special education, family medicine, health administration, nursing, dietetics, and ABA certification.** Several Montana LEND faculty teach within these programs.

In the first year, we will focus recruitment efforts for LTTs on health professions programs on the UM campus in Missoula, which alone accounts for 9 different disciplines. The Montana LEND curriculum is scheduled to fit within the required curriculum of UM health professions programs. Recruitment efforts have already begun and include emails to College of Health faculty and all enrolled health professions students, as well as face-to-face recruitment from Montana LEND faculty within UM health professions programs. In Year 2, we will tap into the additional recruitment pool of local practicing, licensed clinicians. **By Year 3, we will expand recruitment statewide** to include all MUS institutions and clinicians located in other parts of the state. To do so, we will add additional recruitment methods to include information sessions hosted in a hybrid format and exposure to LEND through invitations to short-term training opportunities.

Montana LEND will be able to leverage the strong existing community relationships through UM and other MUS institutions to recruit at least one trainee with a disability and one family trainee each year. Examples of entities within the MUS that have strong existing relationships with Montana's disability community include Montana's UCEDD, on-campus community serving (re)habilitation and wellness clinics, such as the New Directions Wellness Center, UM's Physical Therapy Clinic, DeWit RiteCare Speech, Language, and Hearing Clinic, and the UM Clinical Psychology Center. Additionally, there are organizations in the Missoula area that support families and individuals with autism and other developmental disabilities, including the ARK Foundation, Missoula Circle of Parents, and Opportunity Resources through which we will also share information about Montana LEND and recruit for LTTs.

Retention of LTTs will be achieved through the **payment of a trainee stipend, as well as offering course credits** for completion of Montana LEND didactic components (planned for by Year 3) for enrolled students, which can help them to make progress toward their degree completion. Additionally, the Montana LEND **didactic curriculum will be strategically scheduled** during times that the health professions students do not have required course commitments for their programs. Last, providing hybrid attendance options for didactic components will enable LTTs to more easily participate and complete the training.

There are ample opportunities to recruit medium- and short-term trainees to participate in Montana LEND. As stated in Goal 3.3, we intend to enroll 10 medium- or short-term trainees per year. For short-term trainee opportunities, as described in Goal 2.1, we will offer 4 CE/professional development opportunities annually for licensed and practicing clinicians in Montana to participate in. To increase interest and participation, we will provide opportunities for **travel stipends** for those outside Missoula and **certificates of completion** so that trainees may submit their hours of participation in Montana LEND CE opportunities to count toward CEUs for their professional licensure, as applicable. Additionally, clinicians, health professions students, and individuals with disabilities in the community and their families will be welcomed to engage in regularly occurring didactic curriculum. These **training experiences will be offered in a hybrid format whenever possible**, to increase participation from trainees in more rural and remote areas of the state. To recruit medium-term trainees, enrolled students will be given the opportunity to enroll in course credit for completing Montana LEND didactic curriculum by year 3, ensuring >40 hours of participation.

Training and Education

To best serve trainees from various backgrounds pursuing different disciplines, **the training and education program is designed across didactic/academic (100 hours), leadership (100 hours), and clinical/community domains (100 hours)** to fit both full cohort and individual needs. All trainees proceed together through pre-learning, orientation, and full program activities aimed at providing instruction and guided reflection to ensure all trainees, with their diverse backgrounds, knowledge, and skills, gain a robust understanding of the lifespan developmental approach across a comprehensive curriculum. Finally, students will self-select into teams for a mentored leadership project to allow for practical application of team and leadership skill development, with the goal of supporting sustainable, long-term projects that will serve both trainee and community needs.

Activities and topic areas across each of the academic, clinical/community, and leadership components are geared towards **comprehensive training of an interdisciplinary cohort of students in holistic care of children and youth with autism and other developmental disabilities**. During the needs assessment phase of program development, past LEND trainees representing Montana shared feedback that 100 hours of clinical opportunities are not always a good fit for non-clinical LEND disciplines because clinic observations often focus on clinical practice. For a public health or social work trainee, for example, this amount of clinical practice observation may be out of alignment with their career goals. Based on this feedback, **each trainee will design an individualized plan from a menu of clinical and community-based options**. This ensures a meaningful and tailored experience that will meet their own personal and career goals. The combination of standard (i.e., across all trainees) and personalized (i.e., individual to each trainee) will ensure that trainees with a variety of background skills, knowledge, and experiences can access the curriculum meaningfully to maximize impact.

In the first funding year, the didactic curriculum will begin in late fall and extend into summer term of 2027 to accommodate the September 30 start date. In subsequent years, the curriculum will follow the fall and spring semester schedule. Similarly, the leadership, clinical, and community interdisciplinary experiences will be adjusted to fit the shifted timeline in the first year, with potentially more emphasis on asynchronous opportunities as needed.

Trainees are expected to log their own hours for the domains of leadership and clinical/community experiences. Additionally, trainees should consult with their mentor if questions arise about whether specific activities can count towards hours.

Didactic/Academic Domain

Topics in the didactic curriculum will be delivered by a diverse range of presenters, who will include **clinicians, researchers, affiliate faculty, family members, individuals with disabilities, and community members** (i.e., members of the Rural Institute Community Advisory Council) to best represent varied perspectives and the importance of family- and person-centered approaches. Topics were selected to support a comprehensive training curriculum that also supports the leadership project and the clinical/ community interdisciplinary experiences. A complete list of pre-work, orientation activities, and schedule of topics across the two semesters can be found in *Attachment 5:* Didactic Curriculum Syllabus.

Leadership Domain

In the leadership domain, trainees will gain leadership skills via select orientation activities and participation in activities like conference attendance and community outreach events with Montana LEND faculty. The **primary activity** for leadership skill acquisition will be a mentored team project that focuses on meaningful community impact. After selection into the program and during orientation, trainees will have time to **form teams based on interests, goals, and backgrounds**. During the initial planning phase, teams will coordinate ongoing team meetings and schedule individual work time dedicated to the project to meet the project goals; these hours count towards total 100 hours in this domain. Teams can choose projects from faculty-developed ideas (examples below) or may develop their own. After the first grant year, teams are

encouraged to continue existing projects—new proposals would need longitudinal potential. **Projects that can be sustained through years and cohorts will be prioritized for meaningful impact**; e.g., needs assessment across years, projects that required longitudinal relationships, etc. Each team will be assigned a Montana LEND faculty mentor, though other community members may also be valuable mentors as appropriate. Partnerships with outside organizations are encouraged. Teams will choose their topic, formulate a plan, and then work closely with their project mentor to meet the project goals by the end of the training year. At the conclusion of training, all teams will present their final projects in person at UM. Clinical and community partners, UM faculty, UCEDD staff and Community Advisory Council, and other stakeholders will be invited to attend and support these presentations.

Each year, **one leadership project will focus on development of a continuing education (CE)** experience for Montana providers. LEND trainees in the next cohort will then have the opportunity to present that evidence-based CE training on community outreach trips with Montana LEND faculty. Examples of projects might include, a needs assessment for a Title V partner, quality improvement processes for interdisciplinary clinics, targeted education for providers and/or parents in topics related to autism and developmental disabilities, or conducing a scoping review on a LEND-relevant topic.

To support leadership development, in addition to receiving select instruction within the didactic curriculum, **trainees can accrue additional hours within this leadership domain dedicated to training via required pre-work** (e.g., leadership and communication style assessments, pre-reflection, etc.) and opportunities, including:

- Personal Interdisciplinary Experience Plan (PIEP) development to align with MCH leadership trainings (e.g., plan development, readings, outside trainings, etc.)
- Policy/advocacy training and work to be able to lead this for the communities/ individuals we serve (e.g., Women's Policy Leadership, attending the Disability Policy Seminar, participating in a Montana Legislative Hill Day, etc.)
- Accompanying Montana LEND Faculty on community outreach events in rural and tribal communities. Meeting with potential partners and helping deliver CE and provide TA

Clinical/Community Domain

The PIEP will be developed by all trainees, clinical and non-clinical, at the beginning of the year. The purpose will be to guide trainee participation across domains with specific focus on individualized and rationalized clinical and community hours that intersect with their interest/career development areas. **Trainees will identify specific activities to complete their hours and relate them to goals in the MCH competency areas.** This development will be in conjunction with their faculty mentor and will include a mid-point check-in with their mentor to adjust/modify activities as needed. The creation of an individualized plan facilitates training goals related to ensuring that trainees receive content in ways that are suitable for varying knowledge, skill levels, and backgrounds.

Trainees will use the following activity areas to plan their PIEP with guidance from their assigned mentor. Some experiences will have minimum or maximum time restrictions to guide trainees into a balanced experience. Time spent in collaboration with mentors to develop and adjust the PIEP will be counted towards overall hours.

Clinical observations

Clinical observations may represent the bulk of this experience for clinically focused trainees, though they should not represent more than 90 total hours. Observations must be coordinated with Montana LEND mentors, as well as externship/fieldwork placement coordinators for UM affiliated clinical programs, in order to uphold ongoing relationships with community sites. Many sites already host students as part of agreements for graduate/ professional training, and it is of utmost importance that Montana LEND observations with these clinical partners be coordinated thoughtfully. Trainees must comply with all site requirements when observing in community clinical sites (e.g., background checks, vaccine requirements, etc.). In collaboration with existing clinical programs at UM and the Rural Institute, there are **relationships established with hundreds of sites across the state that could potentially host Montana LEND trainees** for observation.

Media engagement

Trainees can opt to learn from outside the Montana LEND curriculum by reading books, or journals, listening to podcasts, or attending webinars or ECHO sessions. These activities can be done individually or in small groups. They will require a reflection that can be submitted in a variety of formats to account for different learning styles.

Family-directed case consultations

Family-directed case consultations are structured as **interdisciplinary case consultations that give trainees the opportunity to learn directly from families** in a clinically relevant, team-based context. Prior to the session, families or caregivers submit background information and key questions or concerns. An interdisciplinary trainee team is then formed based on the identified needs (e.g., sleep, feeding, school-based concerns, developmental evaluation), and trainees meet in advance to develop recommendations. During the consultation, trainees engage with the family in a one-hour discussion, providing suggestions, asking questions, and learning from the family's experiences. **This model allows trainees to practice discipline-specific roles while collaborating as an interdisciplinary team,** including opportunities for non-clinical trainees to serve as facilitators or note-takers. Emphasis is placed on developing recommendations that are feasible and responsive to the realities of rural and resource-limited settings, including navigating access to care and community supports. To accommodate busy families and those not in Missoula, consultations will be virtual. Faculty mentors will provide real-time coaching to trainees during these experiences to support clinical reasoning and team-based practice. Families receive a stipend.

Family/ home visits

Family partnership visits give trainees the chance to **learn directly from families in real-life settings** and everyday experiences. Visits may happen in the family home, community settings, clinics, schools, or other places chosen by the family. Trainees complete two visits with the same family. Families guide the conversations based on what feels important to them. Trainees are encouraged to come prepared with questions and curiosity, while also being flexible and respectful of the family's comfort and priorities. Trainees are not there to provide clinical care or make medically-based suggestions. Topics of discussion may include **healthcare experiences, school and community supports, disability services, advocacy, transitions, daily life, and the experiences of the whole family**. Families receive a stipend.

Community observations

Trainees may find meaningful learning from observing relevant community activities designed to support individuals with disabilities. Examples might include adapted recreation or exercise classes, work opportunities, library programs, etc.

TeamSTEPPS training

Clinical trainees can participate in full or partial Team Strategies and Tools to Enhance Performance and Patient Safety (TeamSTEPPS) training to bolster communication and teamwork skills in clinical settings.[24,25] TeamSTEPPS is a structured approach for healthcare teams to better improve quality, safety, and efficacy in healthcare, with a goal of improving communication and reducing errors.

Site visits

Trainees can schedule and visit sites that provide non-clinical support, such as providing adaptive technology or other services in the community. Examples in Missoula include MonTECH, the Arc of Western Montana, Aware, and the Learning and Belonging preschool.

Conferences and meetings

Time attending related conferences, trainings, or meetings can count towards clinical/ community experience hours if they are not part of a trainee's usual graduate curriculum. Funding may be available, though there are also several options for free or reduced-rate conferences for students (e.g., MontCOMM, free university conferences, University of Wyoming Project ECHO trainings, etc.)

Trainees may also count time spent at related meetings if invited to participate or if meetings are open to the public, (e.g., the Rural Institute Community Advisory Council meetings, the Native American Maternal Heath Community Advisory Board, etc.).

Shadowing with current research projects

Trainees may observe with related clinical research projects (e.g., assessments, screening, data collection opportunities). For example, the Pediatric Rehabilitation and Active Lifestyle Lab at UM is currently conducting multiple studies focused on improving outcomes in children with motor delays. One is a study on improved function for children with neuromotor disorders using hippotherapy. Another is evaluating a telehealth approach for providing early intervention through a caregiver coaching model. Led by Montana LEND core faculty, Dr. Julia Mazzarella, these studies offer unique opportunities for trainees to experience research and clinical services provided outside of a clinic environment.

Partnerships

In planning for Montana LEND, the leadership team prioritized identifying and confirming key partnerships that will be critical in executing a comprehensive training plan. These partnerships not only include on-campus partners but also partners across the state of Montana. Examples include:

1) Clinical Partnerships

- UM has a strong track record of clinical education in partnership with sites across Montana. Currently, there are **393 established relationships and contracts** in a variety of sites (e.g., hospitals, clinics, early intervention programs, schools, community-based programs). These sites will be reviewed for specifically interdisciplinary experiences and will provide opportunities for trainees to gain valuable clinical experience.

- **Community Medical Center (CMC)** in Missoula, Montana will serve as a key partner in providing experiences in interdisciplinary clinics and services for children with autism and developmental disabilities. CMC employs one of only two board-certified developmental-behavioral pediatricians in the state, who has agreed to join as Montana LEND core faculty. She coordinates **routine, interdisciplinary clinics** that will provide trainees with a wealth of opportunities to observe and participate.
- **Shodair Children's Hospital** specializes in psychiatric care for children and adolescents in addition to having a nationally recognized genetics program for all ages. In addition to their main campus in Helena, they have clinics in Butte and Great Falls which will provide trainees from across the state options for rich clinical experiences. This will specifically **help address the concerns noted in the needs assessment** that CYSHCN who have mental health concerns and/or are dually diagnosed need more support and services from Montana providers.

2) Maternal Child Health State Agencies

- **Montana Autism Education Project (MAEP)** is a statewide program housed in the Office of Public Instruction, Montana's statewide education agency. MAEP provides training and TA specific to supporting children with autism in schools. Montana LEND will partner with MAEP to support their training and TA efforts including providing an annual ADOS-2 clinical training for community members and clinical fellows.
- **Montana Developmental Disabilities Program (DDP)** provides services to individuals with developmental disabilities and has offices across Montana. There are opportunities to partner with DDP for advocacy, leadership projects, community provider training, and TA.
- **Children's Special Health Services (CSHS)** is a program funded by Montana's Title V Maternal and Child Health Services Block grant and focuses on ensuring children and their families receive the healthcare and supportive services they need to lead meaningful lives. They provide education, connect families to resources and provide services such as newborn screenings. This partnership with CSHS will provide our trainees diverse opportunities for **advocacy, training, TA, and information dissemination.**

3) Montana Advocacy Organizations

- **Family Support Services Advisory Council (FSSAC)** is an interagency council that provides input to Montana DPHHS to plan, develop and implement early intervention and family support services for children birth to three with developmental disabilities. Montana LEND will partner with FSSAC to **help address gaps in provider knowledge and use of evidence-based practices through training, TA, advocacy, and information dissemination.**
- **Montana Council on Developmental Disabilities** advocates for individuals and families across Montana to improve services and inclusive communities for persons with developmental disabilities. Working alongside this council, there will be opportunities for **trainee recruitment, advocacy, education, and information dissemination.**

Montana LEND will create partnerships with **community cultural partners including our tribal communities and colleges**. Per focus group feedback that included representatives from the Blackfeet, Rocky Boy, Flathead, Nakoda and Aaniiih, Northern Cheyene, Fort Peck Souix and Bitterroot Salish communities, there are ample opportunities for peer and family support, provider training (specifically centered around decreasing discrimination and improving cultural safety), increasing awareness of

available services, and community/provider education specific to diagnoses including autism and other developmental disabilities.

Lastly, we will **partner with community disability service providers** across the state to establish opportunities for our trainees in addition to determining how Montana LEND can help support their efforts or services. Examples of these include Achievements Inc. (Northwest Montana), the Developmental Educational Assistance Program (Eastern Montana), Opportunity Resources (Missoula), AWARE Inc (locations statewide), and Montana's four Centers for Independent Living serving the entire state.

Technical Assistance and Continuing Education

The Rural Institute is an established and respected provider of both CE and TA on disability-related topics. **In fiscal year 2025, the Rural Institute led over 130 programs or events that trained or educated over 3,500 people.** This included people with disabilities, state and national partners, professionals and paraprofessionals, family members and caregivers, and both legislators and policy makers. Additionally, **in fiscal year 2025 the Rural Institute provided 2,622 documented hours of TA to individuals and organizations across the country.** People seek out Rural Institute staff for their expertise in assistive technology, alternatives to guardianship, youth to adult transitions, navigation of healthcare resources, and self-employment for people with disabilities. All of these topics are closely integrated into the proposed Montana LEND curriculum.

Having Montana LEND housed at the Rural Institute provides Montana LEND faculty with decades of knowledge and applied training experience. As creators of best practices in the provision of accessible content for CE and TA, there will be rich opportunities to collaborate with existing Rural Institute CE and TA programs. Onboarding of LEND faculty will highlight accessible instruction and materials to ensure inclusion for trainees and community organizations.

Many proposed **Montana LEND faculty already provide CE and TA,** including, but not limited to, training and best practices for developmental screenings, lectures in interdisciplinary classrooms and programs, and management of accessible materials and websites. This collective expertise will be organized within a to-be-developed faculty portal to ensure CE and TA requests are directed to the right expert.

The Montana LEND faculty will also commit to providing **much-needed CE specific to evaluation, diagnosis, and treatment of autism and other developmental disabilities**. Each year, the budget will support costs for an in-person workshop from a certified trainer for the Autism Diagnostic Observation Schedule, Second Edition (ADOS-2), including travel stipends for select clinical partners from rural and tribal communities outside of Missoula. In addition, each year, one Montana LEND leadership project will focus on development of an evidence-based CE program related to developmental disability evaluation, diagnosis, or treatment. This will subsequently be offered to Montana providers the following year via at least two trainings facilitated by current LEND trainees (virtual or during in-person outreach events described below). This will facilitate skill acquisition of trainees in content development and delivery, while also building a library of CE specifically designed for Montana providers.

Montana LEND faculty and trainees will coordinate **up to four outreach events outside of Missoula each year**. These events will combine information dissemination,

LEND recruitment, resource sharing, developmental screenings, and CE events. These outreach events will occur **primarily in rural and tribal communities** where in person resources and events are less prevalent than in urban areas. All CE and TA will be logged in a Montana LEND data tracking system overseen by Montana LEND support staff. During these outreach activities, in-person interactions with community members and clinical partners will establish trust and confidence, subsequently leading to future TA encounters with these communities.

High-Level Work Plan

Montana LEND Faculty jointly executed a comprehensive needs assessment to learn directly from individuals with autism and other developmental disabilities, families, providers/clinicians, state agencies, and former LEND trainees about how a LEND program could benefit Montana. Analyzing this information led to the creation of the three overarching goals and subsequent measurable objectives outlined in the *Overall Methodology* section of this proposal. Additional details on specific timelines, responsible parties, and evaluation tools for each objective and underlying activity can be found in the detailed work plan in *Attachment 1.*

The faculty and partners established in this proposal bring a wealth of expertise across a variety of disciplines, including program development, clinical and non-clinical skill building, leadership promotion, high-quality research methods, and information dissemination. Current faculty members have been assigned roles and responsibilities related to their expertise to most effectively plan, design, and carry out the activities listed in the work plan. Future core faculty and affiliate faculty will be identified as program needs develop. **Using the internal evaluation team at the Rural Institute will provide expert-level and consistent feedback to the program.** Each objective and activity will be assessed in a timely manner and modified as needed for the next program year. Having an outside evaluation team will allow the Montana LEND Core Faculty to concentrate fully on the development of each trainee and the holistic delivery of the program.

Resolving Challenges

Montana LEND anticipates several challenges in implementing a comprehensive, interdisciplinary training program across a large, rural, and geographically dispersed state. **The program will proactively address these through structured processes, clear expectations, and continuous quality improvement.**

Trainee Recruitment, Retention, and Engagement

Recruiting and retaining an interdisciplinary cohort of trainees across required disciplines may be challenging due to newness of the program, geographic barriers, and competing academic and clinical demands on trainees and faculty. Additionally, ensuring consistent participation (e.g., accurate tracking of training hours, engagement in extended didactic sessions, and development of a meaningful Personal Interdisciplinary Experience Plan) may present difficulties.

To address these barriers, Montana LEND will implement a **structured recruitment and onboarding process** in collaboration with academic departments at UM, professional organizations, and community partners. Recruitment materials and outreach efforts will emphasize the program leadership’s development focus and

commitment to improving services for children and youth with autism and other developmental disabilities, aligning with LEND's purpose of building workforce capacity.

Retention will be supported through **orientation and expectation-setting**, including a comprehensive Trainee Expectations Guide to be developed by the start of Year 2. Each trainee will develop a Personal Interdisciplinary Experience Plan (PIEP), supported by faculty mentors to ensure their LEND experience is meaningful and feasible. Ongoing communication, cohort-building activities, and individualized mentorship will further support retention across trainees.

To promote engagement, particularly during longer didactic sessions, sessions will **incorporate interactive learning strategies** (e.g., case-based discussions, panels with individuals with lived experience, and applied activities tied to pre-session assignments) consistent with adult learning principles and **universal design for learning.**

Faculty Engagement and Mentorship Capacity

Given the interdisciplinary structure of LEND, variability in faculty availability and competing responsibilities may impact timely mentorship and recruitment for presentations at didactic sessions.

To mitigate these challenges, Montana LEND will **establish clear expectations through a Faculty Handbook** that outlines roles, responsibilities, time commitments, and data documentation requirements. The faculty handbook will be developed by the start of Year 2 and will be informed by lessons learned during implementation of Year 1. Expectations will be reviewed annually during faculty onboarding and reinforced at the first faculty meeting each year. **LEND faculty meetings will occur at least bi-monthly** as an opportunity for core faculty to connect and remain aligned with program goals and priorities. Lastly, faculty will be supported through shared tools, clear data tracking methods, and coordinated schedules to streamline communication and reporting.

Interdisciplinary Training and Leadership Development

Facilitating effective interdisciplinary collaboration and managing trainee group dynamics during each domain of training may present challenges, particularly when trainees have varying levels of experience and come from diverse disciplines and geographic locations. Another potential barrier is ensuring that leadership projects are feasible within the training timeframe, especially when involving community partners.

Montana LEND will address these challenges early in the training year by **providing didactic sessions focused on leadership foundations, teamwork, and interdisciplinary collaboration principles.** During orientation and initial LEND didactic sessions, groups will establish shared goals, roles, and communication expectations.

Leadership projects will be assessed to ensure feasibility within the program year, with options for multi-year continuity as a priority. Dedicated faculty mentorship and structured timelines will support project completion. Community partner engagement will be formalized through clear expectations and ongoing communication.

Clinical Training and Community Partnerships

Providing sufficient clinical experiences across Montana's rural and frontier regions may be challenging due to limited availability of specialty clinics, variability in telehealth use, and competing demands on clinical partners. Ensuring that both clinical and non-clinical trainees have meaningful experiences is also a potential challenge. To address this,

Montana LEND will establish formal and informal agreements with participating clinics that outline expectations for hosting trainees. Additionally, we will **utilize relationships built through existing professional programs** across the MUS system. LEND support staff will manage relationships with clinical sites and serve as a point of contact for representing the clinical experiences associated with the LEND program.

Clinical training will include a **combination of in-person and telehealth observation** opportunities to maximize opportunities access across the state, consistent with LEND expectations to incorporate innovative care delivery models.

To support interdisciplinary learning, trainees will participate in family-centered clinical activities, including family-directed case consultations. **Faculty will provide real-time coaching to trainees** to balance high-quality learning experiences with meaningful service delivery to families. Debrief and feedback will follow each consultation to give the trainees the opportunity to reflect on the interprofessional experience.

Data Tracking, Evaluation, and Continuous Quality Improvement

Ensuring accurate tracking of trainee hours, plus activities and products completed by faculty may present challenges. To address this, Montana LEND will **utilize standardized tracking systems** and provide clear guidance to trainees on documentation expectations. The program will use data to monitor participation, identify emerging challenges, and implement continuous quality improvement strategies, consistent with HRSA expectations for performance measurement and program evaluation. Regular review of feedback (trainees, faculty, and partners) will inform program refinements to ensure that activities remain aligned with program goals and responsive to workforce needs.

Ensuring Relevance to Rural and Indigenous Communities

Delivering culturally and geographically relevant training that reflects Montana's rural and Indigenous contexts may be challenging, particularly in ensuring consistent integration across all program components. Montana LEND will **intentionally embed rural and Indigenous perspectives** throughout the curriculum, including dedicated didactic sessions on Montana context and rural service delivery in Indigenous communities. Panels featuring individuals with lived experience will provide real-world insights and partnerships with community organizations will inform training content and delivery. These efforts will ensure that trainees are prepared to provide culturally and contextually appropriate care.

Performance Reporting and Evaluation

Performance measurement and reporting

Performance measures (PMs) will be tracked and collected by members of the Montana LEND Leadership Team with support from the Evaluation Team. Following HRSA LEND program guidance, we will collect data on the following required PMs:

- Number of long-, medium-, and short-term trainees in Montana LEND.
- Number of CE events and participants.
- Number of TA hours, topics, and recipients.
- Number of diagnostic evaluations conducted by Montana LEND each year.

In addition to the required performance measures, **additional performance measures** that will be tracked and collected will include:

- Number of long-, medium-, and short-term trainees with a disability and/or who are family members of a person with a disability
- Number of LEND faculty who are individuals with lived experience (i.e., family members and individuals with autism/DD)
- Number of disciplines represented by both trainees and LEND faculty
- % of trainees from each discipline
- Demographics (e.g., sex, age, disability, race/ethnicity, etc.) of trainees
- # of partnerships with community clinics, schools, or service agencies that serve as training sites for LEND trainees

The Evaluation Team will review the data for inconsistencies and gaps and work with the Leadership Team to address them. The Evaluation Team will **compile the PM data at least quarterly** into a comprehensive data file that will track PMs over the duration of the grant period. Data will be compiled and shared quarterly with the Leadership Team (see Continuous Quality Improvement section). PMs will also be **reported to HRSA in annual performance measure reports.** The PM data will be used by the Leadership Team to adjust strategies and address any emerging implementation challenges.

We will use AUCD's **National Information and Reporting System (NIRS)** to assist with tracking and reporting on long-term trainees over the five-year grant period. NIRS has robust built-in longitudinal data collection capabilities and the Leadership and Evaluation Teams already have experience using this platform to collect and report information. Strategies that will be used to maintain contact with former trainees include regular outreach to update/maintain current contact information and working closely with the Program Director to follow up with graduates who have been non-responsive.

Data Management and Security

PM data and other quantitative and qualitative data identified in the Program Evaluation Plan below will be managed and securely stored jointly by the Montana LEND Leadership and Evaluation Teams and will be made available to the rest of the Montana LEND team on an as-needed basis. All **individual-level data will be de-identified** before sharing externally, except in instances where identifiable data is required for reporting purposes (e.g., trainee-level data may be shared with HRSA for required annual reporting). Data that may be sensitive, such as feedback on training activities or events, will only be shared in aggregated form to protect participant confidentiality.

All PM and other data associated with the project will be stored in a folder on the University of Montana's secure OneDrive storage, accessible only to the Montana LEND Leadership Team and Evaluation Team. Key personnel on the project (Evaluation Lead—M. Javorka; Program Director—K. McKearnan) will be data stewards while the program is active (i.e., during data collection, coding, analysis, and reporting) and will be responsible for documenting and managing the data throughout this time. Additional project personnel will also be responsible for adhering to the data management plan described above.

We anticipate that this program evaluation will be considered "not human subjects research" pursuant to 45CFR46.102(d) because its intent is to provide information for program evaluation and quality improvement, not to produce generalizable knowledge. Prior to beginning data collection, the Evaluation Team will submit an application to

UM's Institutional Review Board for an official determination of "Not Human Subjects Research." If the IRB determines this to be human subjects research, all applicable human subjects protections, including participant confidentiality, will be followed. All members of the Evaluation Team have completed responsible conduct of research and human subjects training as required by the University of Montana and will follow best practices for the ethical conduct of research and evaluation, regardless of IRB determination.

Program Evaluation

The Montana LEND evaluation will be informed by the **principles of Utilization-Focused Evaluation (U-FE).** U-FEs are designed to ensure that evaluation findings are useful to the primary users and evaluation findings are shared regularly so that implementation can be adjusted to maximize effectiveness and impact.[26] In line with a U-FE approach to evaluation, proposed evaluation activities will be informed by key stakeholders who have been identified as primary users of the evaluation, including the Montana LEND Leadership Team. Along with our internal program evaluation, **Montana LEND will participate fully in the Autism CARES Act evaluation.**

The evaluation will use a **concurrent mixed methods design** in which both quantitative and qualitative data are collected simultaneously throughout the project.[27] Quantitative methods will be used to evaluate the training, events, and other activities provided via Montana LEND, as well as required and additional performance measure tracking as described above. Quantitative data will be supplemented with qualitative information (i.e., key informant interviews and open-ended survey responses) to capture key process and outcome information about program implementation, such as the impact of LEND activities on the system of care.

Primary data collection activities will include:

- Routine performance measure tracking
- Pre- and post-training surveys of trainees to comprehensively evaluate the training curriculum
- Brief post-module surveys of trainees after each didactic module for rapid cycle quality improvement
- Post-session and post-event attendee surveys for CE, professional development, and outreach events
- Annual key informant interviews with program staff, faculty, and partners

Additional information to inform the evaluation may be compiled from program meeting notes (e.g., Leadership Team meetings, faculty meetings) and other programmatic documentation as needed. Below, we present our evaluation questions, data collection activities, and measures, organized by program goal.

Program Goal 1:

Establish and implement a LEND program in Montana that meets the unique needs and attributes of our state by creating the infrastructure, curriculum and partnerships to train and support postgraduate students, family members, individuals with disabilities, and practicing professionals dedicated to improving care for children and youth with autism and other developmental disabilities.

Table 1. Goal 1 Evaluation Questions, Data Collection Activities, and Measures

Evaluation Question	Data Collection Activities	Proposed Measures
1. To what extent has Montana LEND developed the organizational infrastructure for the program, such as faculty, facilities, partner agreements, and virtual platforms?	• Annual key informant interviews • Performance measure tracking	• Qualitative themes related to organizational infrastructure (e.g., successes, challenges) • # of Montana LEND core faculty • # of agreements with facilities and partners
2. To what extent has Montana LEND developed and conducted a community-driven interdisciplinary curriculum in collaboration with core faculty, people with disabilities and their family members, and community partners?	• Annual key informant interviews • Performance measure tracking	• Qualitative themes related to incorporation of lived experience of people with disabilities and their families • # of Montana LEND faculty who are individuals with lived experience of autism or other developmental disability
3. How has Montana LEND increased the number of disability providers, healthcare providers, and community members trained in providing care for children and youth with neurodevelopmental and related disabilities?	• Performance measure tracking	• # of enrolled trainees • % of trainees who complete 2-semester curriculum • # of enrolled clinical fellows • % of clinical fellows who complete 2-semester curriculum and clinical experiences
4. How satisfied are trainees and clinical fellows with training provided through the Montana LEND curriculum?	• Pre- and post-training surveys	• Trainee self-rated knowledge and skill improvements • Trainee satisfaction and training feedback

Program Goal 2:

Provide education, training, technical assistance, and access to evidence-based research and resources to increase Montana provider capability and capacity to evaluate, diagnose and provide evidence-based interventions for children, youth and young adults with autism and other developmental disabilities.

Table 2. Goal 2 Evaluation Questions, Data Collection Activities, and Measures

Evaluation Question	Data Collection Activities	Proposed Measures
1. To what extent has Montana LEND increased CE and professional development opportunities for providers and community members, including autism-specific opportunities?	• Performance measure tracking • Post-session surveys	• # of CE sessions delivered • # and type of CE session attendees • # of other professional development opportunities delivered • # and type of professional development attendees

		• # of autism-specific CE and professional development opportunities • Attendee satisfaction and feedback
2. To what extent does Montana LEND provide technical assistance to disability-related organizations across Montana?	• Performance measure tracking • Program meeting notes & documentation • Annual key informant interviews	• # of TA recipients • # of TA hours provided • # of TA recipients serving rural and tribal areas • Qualitative themes related to provision of TA (e.g., successes and challenges)
3. To what extent has Montana LEND developed and disseminated community-informed leadership projects across Montana?	• Performance measure tracking • Program meeting notes & documentation	• # of leadership projects developed and disseminated • Documentation of community input into leadership projects
4. To what extent has Montana LEND explored opportunities to operate an interdisciplinary clinic for developmental and other disabilities at the University of Montana?	• Program meeting notes& documentation (e.g., proposals, MOUs, agreements)	• Proposal submission to DPHHS RFP for new evaluation and diagnostic clinics in Montana • Establishment of MOUs and other agreements with partners • Development of business plan for clinic sustainability

Program Goal 3:

Develop and strengthen partnerships across Montana, with emphasis in rural, tribal, and frontier communities, to increase access to coordinated, high-quality care and support services for children, youth, and young adults with autism and other developmental disabilities.

Table 3. Goal 3 Evaluation Questions, Data Collection Activities, and Measures

Evaluation Question	Data Collection Activities	Proposed Measures
1. To what extent has Montana LEND established and maintained formal partnerships with providers/clinics, schools, tribal health organizations, Title V funded program, and community groups across rural, tribal, and frontier regions?	• Performance measure tracking • Program meeting notes & documentation • Annual key informant interviews	• # of MOUs or agreements with providers/clinics, schools, tribal health organizations, Title V funded programs, and/or community groups • % of partners serving or located in rural, tribal, or frontier regions • Qualitative themes related to the depth and quality of established partnerships
2. How has Montana LEND increased availability and accessibility of outreach to families and caregivers of	• Performance measure tracking	• #, location, and type of community-based outreach events

people with autism and other developmental disabilities?	• Event evaluations (e.g., brief attendee surveys) • Annual key informant interviews	• Attendee satisfaction and feedback • Qualitative themes related to outreach events (e.g., successes and challenges)
3. How has Montana LEND engaged short- and/or medium-term trainees in didactic training on development and other related disabilities for allied health professionals?	• Performance measure tracking • Annual key informant interviews	• # of short- and/or medium-term trainees participating in didactic Montana LEND sessions • Qualitative themes related to engagement of professional associations and training programs in recruiting short- and medium-term trainees

Evaluation Dissemination

Use of findings from the Montana LEND evaluation will be supported by robust reporting and dissemination tailored to meet the needs of diverse audiences, including program stakeholders, partners, other awardees, and the public.

Annual Evaluation Reporting

The Evaluation Team will compile an annual evaluation report for each year of the grant period to **summarize key performance measures**, short-, medium-, and long-term **trainee feedback**, and results from annual key informant interviews regarding the **quality of TA, partnerships, and outreach** across the state. Annual evaluation reports will assist the Leadership Team with fulfilling annual reporting requirements as well as indicate progress on program goals and objectives. The Evaluation Team will also facilitate annual action-planning workshops based on the previous year's evaluation results to inform programmatic changes and improvements (see Continuous Quality Improvement below).

Summative Report and Dissemination

The Evaluation Team will complete a summative evaluation report in the final grant year to document the extent to which objectives and goals have been met, overall project outcomes, and lessons learned for future programmatic improvement. The summative report will include **analysis of program effectiveness at the local/regional and state levels** to understand program reach and impact across Montana's broad geography, including comparisons of participation, activities, and outcomes across rural and nonrural areas. Drawing on this report, the Evaluation Team will also develop alternative dissemination products for public audiences and other awardees, such as one to two-page evaluation briefs and infographics, to highlight key programmatic outcomes and evaluation findings.

Evaluation Timeline

Evaluation activities will take place regularly over the course of the five-year program, with exact timing of specific data collection activities to be determined based on training, professional development, and outreach schedules. Anticipated timelines for primary data collection and reporting activities can be found in the table below.

Evaluation Activity	Anticipated Timeline

Performance measure tracking	Compiled quarterly, beginning in Q3 of Year 1
Pre- and post-training surveys of trainees	- Pre-training survey administered as part of enrollment - Post-training surveys administered after each didactic module of the 2-semester training curriculum to inform modifications to the following module - Final cumulative evaluation survey administered upon completion of training curriculum or exit from program
Post-session/post-event attendee surveys	Administered during or immediately following CE, professional development, and outreach events
Key informant interviews with program staff, faculty, and partners	Conducted annually in Q3/Q4, beginning in Year 1
Annual evaluation report and action planning	Compiled/conducted in Q1 of each year for the previous year, beginning in Year 2
Summative evaluation report and dissemination activities	End of grant period

Anticipated Evaluation Barriers

The Montana LEND evaluation is designed to be adaptive and responsive to the development of a new program, including unanticipated implementation challenges. However, several barriers are anticipated based on our prior experiences with other LEND programs and evaluations. Proactively identifying these potential barriers will allow the Evaluation Team to prepare mitigation strategies and ensure the evaluation remains feasible, ethical, and useful over time.

Trainee Follow-up and Attrition

Maintaining contact with long-term trainees throughout their training and following completion may be difficult, particularly due to contact information/email address changes. This could impact the completeness and accuracy of outcome information.

> **Mitigation strategy:** Ensure regular attempts are made by the Data and Communications Coordinator to stay in touch with long-term trainees during and after training, with explicit requests to update trainee contact information. Compile multiple forms of contact information (i.e., phone numbers, email addresses, and mailing addresses) during program enrollment and attempt to contact trainees through multiple modalities if necessary.

Staff and Partner Capacity

The Montana LEND implementation team, faculty, and program partners will be balancing multiple responsibilities with limited staff capacity. Participating in evaluation activities (e.g., surveys, interviews, data tracking) may create additional demands.

> **Mitigation Strategy:** The Evaluation Team will streamline data collection tools to minimize burden, align evaluation tasks with existing workflows where possible, and clearly communicate the purpose and value of each activity. Faculty and partners will be consulted to ensure that expectations for participation in annual key informant interviews and other evaluation activities are realistic and well-timed.

Establishment of New Program and Evolving Program Strategies

As a newly established program, the evaluation needs and capacity of Montana LEND may evolve in response to program timelines and staffing, partner and community needs, or funding changes. These changes may affect evaluation priorities or the availability of certain types of data.

> **Mitigation Strategy:** The evaluation plan will remain flexible, with regular review and revision points built in. The Evaluation Team will use developmental evaluation strategies to adjust focus as the program evolves and ensure continued alignment with implementation goals.

Evaluation Capacity

The Rural Institute Evaluation Team has experience applying a variety of research and evaluation methods, with a particular emphasis on community-engaged and mixed methods approaches. We also have expertise in disability services, maternal and child health, early childhood education and intervention, rural health, and healthcare systems.

Evaluation Lead

Dr. McKenzie Javorka will serve as the Evaluation Lead for the Montana LEND evaluation. She holds a PhD in community psychology and is experienced in conducting mixed methods community-engaged research and evaluation. Her research has focused on social determinants of health, rural healthcare systems, sexual health and infectious disease prevention, and maternal and child health. She has led and contributed to numerous program evaluation and needs assessment projects, including leading the evaluation of Montana's HRSA Advanced Nursing Education – Sexual Assault Nurse Examiner training program. She has experience leading and training teams on effective qualitative data analysis, data visualization, and data dashboards.

Program Evaluator

Dr. Elizabeth Sabbath holds a PhD in Sociology with a specialization in quantitative research methods and statistical data analysis. Her background includes working with large administrative datasets and applying advanced statistical techniques—including regression, longitudinal panel modeling, and geospatial analysis—to examine complex health and social issues. Her prior work includes serving as a research assistant on a multi-site National Science Foundation study involving crime data analysis from over 90 U.S. police departments. Dr. Sabbath will support data collection, analysis, and reporting for Montana LEND.

Continuous Quality Improvement

The Evaluation Team will use the **Rapid Cycle Quality Improvement (RCQI) model** to provide real-time feedback to the Leadership Team regarding trainee needs and experiences, including information on whether and how Montana LEND is meeting the needs of the disability-serving workforce throughout the five-year program.[28] Performance measures and evaluation findings will be regularly shared with the Leadership Team to support continuous quality improvement and programmatic decision-making. The Evaluation Team will support the Leadership Team in interpretation and use of data by using the **ORID (Observe, Reflect, Interpret, Decide) framework** to facilitate group discussions and action planning sessions.[29]

RCQI will be supported by development of simple data dashboards and infographic summaries, allowing for effective data visualization of performance measures that can

be shared with the Leadership Team on a regular basis throughout the implementation period. These data dashboards will provide digestible data summaries to **show progress on program goals, objectives, and activities**. The Evaluation Team and Leadership Team will collaboratively establish a feasible and useful cadence (e.g., quarterly) for providing updated dashboards. Where appropriate, dashboards will **include comparisons by region and rurality** (i.e., rural vs. nonrural areas) to assess program reach and effectiveness at both the local/regional and state levels.

In addition to performance measure data dashboards, the Evaluation Team will provide **summaries of post-training survey results immediately after each didactic training module** is completed, which will be used by the Leadership Team and faculty to respond in real time to trainee feedback and needs. Results will be shared in accessible, easy-to-digest formats, informed by best practices for data visualization.

To ensure **quality of CE, professional development, TA, and outreach events**, the Evaluation Team will also produce routine evaluation summaries that compile attendee survey results. As with post-training summaries, these will be shared in a brief, easy-to-digest format to facilitate use and action planning by the Leadership Team.

Action planning sessions will be held with the Montana LEND Leadership Team at least annually, following the completion of the annual evaluation report. These sessions will be facilitated by the Evaluation Team with the purpose of using the prior year's evaluation findings to improve program processes and outcomes. Additional action planning sessions may be held on an as-needed basis to address implementation challenges as they arise.

Sustainability

Montana LEND is committed to sustaining the work developed and implemented beyond the grant period. The program is a vibrant opportunity to train future healthcare providers and build networks of professionals trained to view autism/DD services through a multi-disciplinary lens. We plan to respond to notice of funding opportunities for LEND as long as they are available. However, continuation and growth of this work beyond the grant period is critical to the long-term support of children, youth and adults with neurodevelopmental disabilities in our rural and frontier state. Our project sustainability plan is as follows.

A significant number of the proposed Montana LEND faculty were past LEND trainees and fellows. These faculty are now established in Montana, committed to the project, and connected to a significant number of resources throughout the state. Faculty will use their knowledge of Montana resources to inform and refine program development, resulting in **trainees and community partners forming a network that disseminates LEND and LEND-related knowledge throughout the state** for years to come.

Key sustainability components include the **long-term preservation of, and continued dissemination of CE and professional development activities** created during this LEND cycle. The Rural Institute has avenues already created for hosting and disseminating this type of information. Montana LEND and partners will also **maintain and disseminate leadership projects** and use these project-based foundations for **leveraging additional funding** from regional, state, and federal grantees.

To **expand funding** that can further support Montana LEND objectives, the team will lean on the Rural Institute and core faculty's proven history of successful grant funding

to find new or collaborative opportunities. The Rural institute has successfully held a Rural-focused Research and Training Center grant from the National Institute on Disability, Independent Living, and Rehabilitation Research for over 35 years and the statewide assistive technology program funding for over 30 years. It also houses the Montana Family to Family Health Information Center and contracts with DPHHS' Children's Special Health Services to facilitate transition resources for CYSHCN. These programs align naturally with the mission and goals of the Montana LEND and would make stellar partners for future grant-funded work. Additionally, Montana's DPHHS plans to release a request for proposals to fund evaluation and diagnostic clinics across the state in fall 2026. Montana LEND, in partnership with UM and the Rural Institute, will **apply for an autism evaluation and diagnostic clinic** to be opened on UM's campus.

Montana LEND anticipates several challenges to sustaining program activities beyond the federal funding period, particularly given the state's rural context. Montana continues to face significant workforce shortages in providers trained to deliver interdisciplinary, evidence-based care for children and youth with autism and other developmental disabilities, especially in rural and tribal communities. Staff turnover and limited access to specialized training opportunities may impact the long-term availability of trained professionals. Financial sustainability also presents a challenge, as Montana has a limited funding base and high reliance on time-limited grant funding to support workforce development, training, and system improvement initiatives. Additionally, Montana's large geographic area creates ongoing barriers to accessing local providers and specialists, as evidenced by the results of the needs assessment. Sustaining meaningful partner engagement across diverse systems—including healthcare, education, tribal health, and community-based organizations—may also become more difficult without dedicated funding and infrastructure.

Montana LEND will address these challenges with a **multifaceted sustainability plan focused on capacity-building, systems integration, and diverse funding.** The program will establish a strong workforce pipeline through a **train-the-trainer model**, equipping LEND graduates, clinical fellows, and partner providers to serve as future trainers and mentors within their communities. Ongoing engagement with alumni through communities of practice and CE will reinforce skills and sustain interdisciplinary collaboration. To support financial sustainability, Montana LEND will continue to **pursue federal funding opportunities** while leveraging the Rural Institute and UM's demonstrated success in securing competitive grants. The program will actively **pursue state funding opportunities**, including DPHHS' initiative to expand autism evaluation and diagnostic services, and explore revenue-generating models such as fee-based training and TA. Montana LEND will also **maximize the use of telehealth and virtual learning platforms** to overcome geographic barriers, ensuring statewide access to training and consultation. **Formal partnerships will be sustained through memoranda of understanding and aligned goals**, fostering shared ownership and long-term collaboration. Finally, a robust evaluation framework will be used to monitor outcomes, maintain fidelity, and demonstrate value to stakeholders, supporting continued investment and integration of LEND practices into statewide systems of care.

The Montana LEND team is invested in sharing information about Montana's LEND and the LEND program in general with groups in-state and nationally. Our team will achieve this with the collaboration of the Rural Institute's communication manager and MT

LEND support staff, who will support outreach and communication of grant activities and information about the program.

A core component of Montana LEND's evaluation plan is ensuring that program outcomes translate into measurable improvements in the system of care for children and youth with autism and other developmental disabilities. Evaluation findings will be used to guide **continuous quality improvement, demonstrate program effectiveness to stakeholders, and support sustainability through data-driven funding and policy decisions.**

Montana LEND will have a substantial and lasting public health impact by strengthening the statewide system of care for children, youth, and young adults with autism and other developmental disabilities. By expanding the capacity of an interdisciplinary workforce trained in culturally responsive, evidence-based practices, the program will improve access to high-quality services, particularly in rural, tribal, and underserved communities. Increased provider training and the development of interdisciplinary clinical capacity will support earlier identification and diagnosis, reduce delays accessing critical services, and improve developmental and functional outcomes. Through strong partnerships and coordinated care approaches, Montana LEND will enhance system integration across healthcare, education, and community services, resulting in more efficient and effective support for individuals and families. The program's emphasis on inclusion of family members and individuals with lived experience will further **ensure that services are responsive and aligned with community needs.** Collectively, these efforts will contribute to improved health outcomes, reduced disparities, and a more sustainable, responsive system of care across the state of Montana.

Organizational Information

UM is a public flagship research university in Missoula, Montana. With over 500 degree pathways and a commitment to public research, UM delivers well-rounded educational experiences grounded in service learning and evidence-based academic rigor. In 2022, UM earned "R1" status as a research institution from the Carnegie Classification of Institutes of Higher Education organization.

Across campus, there are programs, departments, and colleges poised to collaborate with Montana LEND and ready to help recruit and retain students. UM's College of Health is home to degree-granting programs including Pharmacy Practice, Physical Therapy, Physician Associate, Public & Community Health Sciences, Social Work, and Speech, Language, Hearing, & Occupational Sciences. Four core faculty hold academic appointments in the College of Health. Colleagues in the Phyllis J. Washington College of Education partner closely with both the College of Health and the Rural Institute for Inclusive Communities. The chair of the College of Education has joined the team as Affiliate Faculty for Special Education. UM also supports the Family Medicine Residency of Western Montana, affiliated with the University of Washington Family Medicine Residency Network. This network exists to provide superior, comprehensive education in Family Medicine and is committed to developing physicians motivated to serve patients in rural and underserved areas of Montana. This overlap with the Montana LEND mission provides great opportunities for partnerships and collaborations.

Beyond UM, existing partnerships between Montana LEND faculty and Montana State University, Montana State University-Billings, Rocky Mountain College, and Montana's seven Tribal Colleges provide a solid foundation for Montana LEND to expand.

The Rural Institute for Inclusive Communities is the state's premier research, service, and training center on disability. **As a UCEDD, the Rural Institute oversees over 50 programs and projects, all centered on improving access and opportunities for people with disabilities.** The core functions of a UCEDD are to conduct research and evaluation, provide direct service, lead education and training activities, and disseminate information widely – all activities that also fit into a LEND curriculum. The Rural Institute has **operated since 1978** on UM's campus, serving Montanans with disabilities across the lifespan. The Rural Institute has been a supporter and partner of the Utah Regional LEND (URLEND) program as a partner state for decades. In 2023, Montana LEND core faculty approached URLEND about our intentions to apply for a standalone program. This decision was met with encouragement and support from URLEND. **Montana's participation as a partner state ended amicably at the end of the 2025-2026 training year.**

The Montana LEND core faculty includes representation from across the UM Campus and will be jointly supported by existing UM and Rural Institute entities. The Rural Institute employs a business team, IT staff, communications manager, and accessible media specialist who are available to assist with general technical operations of Montana LEND alongside the core faculty. This team is well versed in the management of dozens of complex grants and will support the PIs/PDs to successfully oversee the Montana LEND operations. UM's **Office of Sponsored Programs** employs a dedicated post-award specialist to conduct fiscal oversight, billing, and ensure accurate recording of funds. A to-be-hired Montana **LEND Data and Communications Coordinator** will join the team to work under the direction of the lead PI/PD.

The Rural Institute has a rich history of providing accessible content in accessible spaces. Staff are engaged in ongoing training to ensure documents, dissemination materials, and presentations are inclusive for all audiences. One of the Rural Institute's programs is **Montana's assistive technology (AT) statewide program, MonTECH**. MonTECH is well-versed in the latest technology for hybrid service and information delivery and will be engaged with Montana LEND to ensure the most up-to-date technology is available for optimal trainee participation.

Another program of the Rural Institute is the **Montana Family to Family Health Information Center (MT F2F)**. The MT F2F has been a go-to resource for families seeking assistance and support for their children and youth with healthcare needs. They have successfully provided training and TA to families, service providers, interdisciplinary clinicians, and state employees since 2019. They are well-versed in the urgent and emerging needs of Montana families and will be a valuable resource to Montana LEND.

Project Director, LEND Faculty, and staff qualifications

Leadership Team

Principal Investigator/Program Director: **Dr. Kim McKearnan** graduated from the University of Puget Sound with her OT degree. She subsequently earned a master's degree in physical medicine and rehabilitation (an OT/PT combined program) and a

PhD in nursing science, both from the University of Washington. She has spent her career caring for children in a variety of settings including outpatient specialty clinics and inpatient acute care. This includes nearly two decades of caring for children with autism and other developmental disabilities. She is the founding director of the entry-level occupational therapy doctorate program at UM. Kim has a successful record of program development and cultivating high-functioning teams which will benefit the initiation and sustainability of a LEND program. As a former short-term LEND trainee at the University of Washington's Center on Human Development and Disability and LEND core faculty at Oregon Health and Science University's (OHSU) Institute on Development and Disability, she is deeply committed to LEND's mission to improve the health and well-being of children with disabilities. Given her diverse educational and clinical background, Kim is invested in interdisciplinary and family-centered care.

Co-Principal Investigator: **Molly Kimmel** graduated from Gonzaga University and had an early career in adult education before pursuing OT as an avenue to help people more fully participate in meaningful and necessary activities. She received her master's degree in OT from the University of Washington and practiced at Providence St. Patrick Hospital in Missoula for 11 years. Molly transitioned to the role of Program Director at MonTECH, Montana's statewide assistive technology program, in April of 2020. In addition to her role at MonTECH, Molly was the Montana State Coordinator and OT faculty for URLEND since completing the fellowship herself in 2018, transitioning off faculty in 2024. Molly became the Director of MonTECH's parent organization, the Rural Institute for Inclusive Communities in 2024. She enjoys working on a united mission of building access and opportunity for people with disabilities through education, research, evaluation, and service, managing several large-scale grants and contracts. Molly is passionate about neurodevelopmental rehabilitation, AT, building connections to provide best practices in care, and helping people achieve greater independence.

Co-Program Director: **Julia Mazzarella** holds a Doctor of Physical Therapy degree and a PhD in Health and Rehabilitation Science from The Ohio State University (OSU). Julia completed LEND training as a long-term trainee through OSU's Nisonger Center in 2019, then continued with the program for a second year as an advanced trainee, where she provided teaching assistance and mentorship to the LTTs. She is currently an Assistant Professor in the School of Physical Therapy and Rehabilitation Science at UM and a licensed and practicing physical therapist with ongoing involvement with interdisciplinary clinics. Julia is the director of a research lab, the Pediatric Rehabilitation and Active Lifestyles (PALS) Lab, which focuses on developing and evaluating fun, engaging, and effective PT interventions for children with neuromotor disorders to improve function and participation. Julia is passionate about family-centered care and improved care for children with disabilities. Her ongoing activities align well with the LEND mission, and the community partnerships she has cultivated through her research will provide excellent opportunities for Montana LEND trainees.

Family Faculty: **Jennifer Banna** earned a bachelor's degrees in Child Development/Family Science and Elementary Education from Montana State University. As the parent and grandparent of children and youth with health care needs and disabilities, she brings extensive lived experience navigating health care, education, and disability systems. She currently serves as Center Coordinator for the statewide Montana Family to Family Health Information Center at the UM's Rural Institute for

Inclusive Communities, where she supports families and professionals in navigating complex systems and accessing services and supports across Montana. As a former Montana-based participant in URLEND Family Trainee, Jennifer is committed to interdisciplinary and family-centered approaches that improve outcomes for children and youth with developmental disabilities and their families. As Family Faculty, Jennifer will work closely with our faculty with lived experience to ensure trainees with experience of disability feel included and central to the program.

See the *Evaluation Capacity* section for detailed information on key Evaluation Staff.

Core Faculty

Additional Core Faculty will assist with programmatic elements, including developing and delivering didactics, mentoring trainees, facilitating clinical and non-clinical experiences, and helping with overall program management. These committed Core Faculty include **Drew Babcock**, the Indigenous Health & Curriculum Specialist for the Family Medicine Residency of Western Montana and co-chair of the Montana LEND Curriculum Committee; **Luke Santore**, a Sociologist and Research Associate at the Rural Institute who brings lived experience with a disability to his role with Montana LEND; **Krys Standley**, a former URLEND trainee, Certified Health Education Specialist, and Project Director at the Rural Institute who will lead Public Health trainees; **Leah Meloy**, former OHSU LEND fellow and current Clinic Director for the School of Speech, Hearing, and Occupational Sciences, who will be Clinical Lead and Core Faculty for SLP trainees; **Bonnie Stephens**, a board certified developmental-behavioral pediatrician who will be pediatric medicine Core Faculty; **Polly Becker**, a MSW with more than 25 years of experience working children in a variety of settings will be the Social Work Core Faculty; and **Ginger Sillars,** a certified pediatric nurse with both clinical and teaching experience will be Nursing Core Faculty.

Affiliate Faculty

Affiliate faculty will not receive compensation on the grant but have agreed to assist with didactic seminars in their topics of expertise and meet with trainees to help develop their PEIPs. These include **Morgan Alwell** (Affiliate SPED Faculty/current Chair and Professor of the Department of Teaching and Learning in the College of Education and special education expert), **Anoop Basavanahalli Jagadeesh** (Affiliate Audiology Faculty and Assistant Professor in the School of Speech, Language, Hearing and Occupational Sciences), and **Amy Violante** (Affiliate School Psychology Faculty/current Licensed Psychologist, Nationally Certified School Psychologist, and a Clinical Assistant Professor of School Psychology). As the LEND program progresses through Years 2 – 5, the Leadership Team will onboard additional Core and Affiliate Faculty to meet evolving program needs.

Montana LEND Support

The Rural Institute for Inclusive Communities will provide general budgeting assistance and oversight from their business office, along with support from their dedicated IT professional and internal Data and Communication Coordinator. If funded, Montana LEND will hire an additional .5 FTE **Data and Communication Coordinator** to manage all data input, data systems, reporting, trainee communications, and to act as a clinical liaison.

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Activity	Timeline	Responsible Entity
Goal 1: Establish and implement a LEND program in Montana that meets the unique needs and attributes of our state by creating the infrastructure, curriculum, and partnerships to train and support postgraduate students, family members, individuals with disabilities, and practicing professionals dedicated to improving care for children and youth with autism and other developmental disabilities.		
Objective 1.1: Implement the organizational infrastructure for a Montana LEND program, including faculty, facilities, partner and/or clinical experience agreements, virtual education or service platform, and administrative services. Objective 1.1 **Outputs**: Minimum 7 interdisciplinary faculty team recruited and onboarded annually; administrative staff hired; dedicated hybrid training space and equipment established; virtual learning platform launched with curriculum, trainee tracking, and participation monitoring systems. Objective 1.1 **Outcomes**: Montana LEND establishes the organizational infrastructure needed to deliver a sustainable, high-quality, interdisciplinary training program, including systems for curriculum delivery, participation tracking, and program administration.		
Activity 1.1.1 Recruit and onboard at least seven faculty from different disciplines each year	Ongoing	Leadership Team (LT)
Activity 1.1.2 Secure consistent space and equipment for hybrid didactic sessions	Year 1 (Y1)	PI/PD
Activity 1.1.3 Hire, onboard, and retain .5 FTE Data and Communications staff	Quarter 1 (Q1), Y1	PI/PD
Activity 1.1.4 Develop, integrate, and operationalize curriculum within a virtual education platform, including creation of systems to monitor and document trainee participation and training hours	Y1	All faculty
Objective 1.2: Develop, conduct, and evaluate a 2-semester (academic year) community-driven interdisciplinary curriculum for the didactic, clinical, and leadership/research aspects of the Montana LEND program in collaboration with core faculty (including a self-advocate and a family member), community partners, and related staff. Objective 1.2 **Outputs**: Comprehensive, community-informed 2-semester curriculum developed and updated annually; minimum two faculty members with lived experience of developmental disability (one person with a disability and one family member) recruited and onboarded; annual evaluation reports and final summative evaluation report completed. Objective 1.2 **Outcomes**: Trainees demonstrate increased knowledge and competencies in family-centered, culturally and linguistically appropriate diagnostic evaluations and evidence-based interventions for children and youth with developmental and related disabilities; evaluation findings are used to continuously improve curriculum quality, relevance, and responsiveness to community needs; the number of diagnostic evaluations provided by LEND trainees is increased.		
Activity 1.2.1 Build a comprehensive LEND Curriculum, to be modified yearly based on feedback and evaluations	Q1, Y1, then ongoing	LT, curriculum co-chairs
Activity 1.2.2 Recruit and onboard one family faculty member and one faculty member with lived experience of autism or other developmental disability	Ongoing	LT

Activity 1.2.3 Fully execute the comprehensive evaluation plan to monitor and assess all program activities	Ongoing	Eval Team
Objective 1.3: Recruit nine multi-disciplinary trainees yearly from across Montana, including one family member and one individual with autism or other developmental disability by year two. Objective 1.3 **Outputs**: Nine long-term interdisciplinary trainees enrolled annually; family member trainee and trainee with lived experience of developmental disability participating annually; individualized trainee learning plans (PIEPs), mentorship, and retention supports implemented. Objective 1.3 **Outcomes**: Montana LEND recruits and retains a cohort of interdisciplinary trainees from across Montana that reflects the perspectives of families, individuals with lived experience, and multiple professions.		
Activity 1.3.1 Utilize robust recruitment plan (see page 10) to fill nine trainee spots yearly	Q1, Y1, then Q3/Q4 Y2-5	All faculty
Activity 1.3.2 Support participation for one family member trainee and one trainee with lived experience of autism or other developmental disability yearly	Y2-5	All faculty
Activity 1.3.3 Use strategic tools to maintain trainee retention, including expectation setting, PIEP development, and faculty mentorship	Build in Y1, modify Y2-Y5	All faculty
Activity 1.3.3 Seek and respond to feedback from Montana professional training programs and community workplaces to ensure design and schedule of LEND program is accessible for trainees studying and working in Montana	Ongoing	LT/Eval Team
Objective 1.4: By year three, implement a clinical fellow traineeship for at least one trainee that will be comprised of the standard Montana LEND curriculum in addition to further training and clinical experiences focused on working with children, youth and young adults with autism and other developmental disabilities. Objective 1.4 **Outputs**: Clinical fellowship infrastructure established (space, supervision, clinical experiences) and recruitment process developed; one clinical fellow successfully completes the fellowship annually beginning Year 3. Objective 1.4 **Outcomes**: Clinical fellows develop advanced competencies in evaluation, diagnosis, and intervention for children and youth with autism and other developmental disabilities; the number of diagnostic evaluations provided by clinical fellows is increased.		
Activity 1.4.1 Identify appropriate space and supervision for clinical fellow activities	Q4, Y2	Clinical Faculty/LT
Activity 1.4.2 Recruit within programs with clinical fellowship requirements to identify trainees who would most benefit from and contribute to this focused traineeship	Q3/Q4, Y2 – Y5	All faculty
Activity 1.4.3 Graduate a clinical fellow yearly beginning Year 3	Y3 – Y5	All faculty
Goal 2: Provide education, training, technical assistance (TA), and access to evidence-based research and resources to increase Montana provider capability and capacity to evaluate, diagnose and provide evidence-based interventions for children, youth and young adults with autism and other developmental disabilities.		

Objective 2.1: Organize and deliver at least four continuing education (CE) and/or professional development opportunities yearly for healthcare and disability providers and community members across Montana, developed by LEND faculty, trainees, and/or clinical partners Objective 2.1 **Outputs**: Minimum of four CE/professional development events delivered and documented annually: ADOS-2 training delivered yearly for community partners; Autism Community of Practice established by Year 2; one leadership project on CE developed annually; minimum two out of state outreach trips conducted annually. Objective 2.1 **Outcomes**: Healthcare and disability providers and other community professionals increase their knowledge and confidence in diagnostic evaluation and evidence-based interventions for autism and developmental disabilities; Statewide access to CE and professional development related to developmental and related disabilities is expanded.		
Activity 2.1.1 Plan, promote, and deliver ADOS-2 training for community partners once yearly	Q4, Y1-Y5	Data and Communications Coordinator (DCC)
Activity 2.1.2 By year 2, host or co-host an Autism-specific community of practice or ECHO with mentoring opportunities for trainees and community members	Y2 – Y5	Clinical faculty
Activity 2.1.3 Create and oversee one leadership project focused on development of a CE activity yearly	Ongoing	All faculty
Activity 2.1.4 Arrange at least two outreach trips outside Missoula yearly that include a CE component	Ongoing	LT
Activity 2.1.5 Effectively log CE events by all LEND faculty	Ongoing	DCC
Objective 2.2: Montana LEND faculty, staff, and clinical partners will serve as a valued resource for TA to at least 15 disability-related organizations across Montana, including to rural and tribal communities Objective 2.2 **Outputs**: CE/TA portal developed and maintained; TA promotional/marketing materials developed (web and print); TA provided to at least 15 organizations annually and documented. Objective 2.2 **Outcomes**: Organizations receiving TA increase their capacity to implement evidence-based, family-centered, and culturally appropriate practices for children with autism or developmental disabilities and their families: Montana providers and disability-related organizations increasingly recognize Montana LEND as a trusted resource for expertise in autism and developmental disabilities.		
Activity 2.2.1 Create and maintain CE/TA portal to map subject matter expertise among LEND faculty and recent trainee graduates	Build in Y1, then ongoing	DCC
Activity 2.2.2 Promote and market TA expertise during outreach events and via website	Ongoing	DCC
Activity 2.2.3 Effectively log TA activities by all LEND Faculty	Ongoing	All faculty
Objective 2.3: At least three leadership projects will be developed yearly using community input and disseminated for use across the state upon completion. Objective 2.3 **Outputs**: Three community-informed leadership projects completed annually: leadership projects presented at annual community symposium and disseminated statewide.		

Objective 2.3 **Outcomes**: Trainees strengthen leadership skills through completion of community-informed projects; leadership projects address identified community priorities and produce practical resources that support providers, families, and organizations across Montana.		
Activity 2.3.1 Use needs assessment data to create leadership projects	Q4, Y1 – Y5	All faculty/eval team
Activity 2.3.2 Collect project ideas during outreach events and via clinical partner survey	Ongoing	All faculty/eval team
Activity 2.3.3 Support trainee involvement with regular faculty mentor meetings for effective project development	Ongoing	Faculty project mentors
Activity 2.3.4 Present completed leadership projects at community symposium	Q4, Y1-Y5	LT/DCC
Objective 2.4: In partnership with the University of Montana and Montana Department of Public Health and Human Services, use leveraged funding to open and operate an interdisciplinary evaluation, diagnostic, and treatment clinic on the University of Montana campus by Year 3. Objective 2.4 **Outputs**: Proposal submitted for evaluation, diagnostic, and treatment clinic funding; clinical space and equipment identified through university partnerships; comprehensive clinic business and sustainability plan completed. Objective 2.4 **Outcomes**: Establish the organizational readiness and capacity necessary to launch a sustainable interdisciplinary clinic for the evaluation, diagnosis, and treatment of autism and other developmental disabilities on the University of Montana campus.		
Activity 2.4.1 Respond to DPHHS request for proposal to fund new evaluation and diagnostic clinics in Montana	Q1/Q2, Y1	LT
Activity 2.4.2 Secure space and equipment via partnerships with other UM College of Health programs	Y2	LT
Activity 2.4.3 Develop full business plan for sustainability	Y2	LT
Goal 3: Develop and strengthen partnerships across Montana, with emphasis in rural, tribal, and frontier communities, to increase access to coordinated, high-quality care and support services for children, youth, and young adults with autism and other developmental disabilities.		
Objective 3.1: By year three, MT LEND will establish and maintain a minimum of eight formal partnerships (MOUs or agreements) with providers/clinics, schools, tribal health organizations, Title V funded programs and community groups across rural, tribal, and frontier regions for meaningful collaboration with Montana LEND. Objective 3.1 **Outputs**: Three new partnership agreements/MOUs executed annually, with at least eight formal partnerships established by Year 3; at least one partnership established in each region of Montana by Year 3; Tribal cultural brokers identified and at least one tribal partnership established by Year 3. Objective 3.1 **Outcomes**: Rural, tribal, frontier, and community organizations actively engage in Montana LEND planning, training, and workforce development activities: formal partnerships increase opportunities for interdisciplinary training, clinical experiences, and collaborative service delivery across Montana.		

Activity 3.1.1 Obtain three fully executed MOUs yearly	Q1, Y1 – Y5	LT
Activity 3.1.2 Ensure partnerships reflect diversity of Montana regions	Ongoing	LT
Activity 3.1.3 Identify tribal cultural brokers to guide partnerships with tribal groups and continue ongoing needs assessments for meaningful training and collaboration	Initiate Y1, then ongoing	LT/curriculum co-chair
Objective 3.2: Host at least four community-based outreach events per year in a combination of rural, tribal, and urban communities, reaching at least 100 families and caregivers yearly. Objective 3.2 **Outputs**: Minimum of four outreach events conducted annually across rural, tribal, and urban communities, with at least one event being virtual: minimum 25 attendees (families/caregivers) enrolled per event: Montana LEND and partner resources distributed at each event. Objective 3.2 **Outcomes**: Families, caregivers, and community members increase awareness of available resources, supports, and evidence-based information related to autism and developmental disabilities.		
Activity 3.2.1 Enroll 25 attendees at each event	One/quarter, Y1–Y5	All faculty
Activity 3.2.2 Disseminate topical MT LEND, Rural Institute, and UM resources	Q4, Y1 – Y5	DCC
Activity 3.2.3 Provide a combination of virtual and in-person offerings to maximize reach in rural communities	Ongoing	All faculty
Objective 3.3: Engage at least 10 short- and/or medium-term trainees from across Montana yearly who participate in didactic sessions eligible for CE credit for allied health professionals. Objective 3.3 **Outputs**: Minimum ten short-/medium-term trainees participate annually; CE opportunities promoted through statewide professional networks; faculty leadership assigned for short-/medium-term trainee coordination and documentation. Objective 3.3 **Outcomes**: Short- and medium-term trainees increase their knowledge of evidence-based evaluation, diagnosis, and intervention for children with autism and developmental disabilities and their families through participation in Montana LEND educational activities; the number of providers in allied health professions trained to screen, evaluate, diagnose, and provide evidence-based interventions to children with autism and developmental disabilities is increased; the number of diagnostic evaluations provided by LEND trainees is increased.		
Activity 3.3.1 Advertise through state professional associations and past LEND trainees	Ongoing	DCC/All faculty
Activity 3.3.2 Continue ongoing coordination with professional training programs and community sites to engage trainees in CE that would meaningfully supplement their current training and practice	Ongoing	All faculty
Activity 3.3.3 Identify 1-2 faculty members to lead short- and medium-term trainee experiences and ensure accurate data collection	Y1, then ongoing	Faculty leads

Attachment 2: Staffing plan and job descriptions

In late 2023, a group of 6 former LEND trainees from across the University of Montana met to discuss the feasibility of starting a Montana LEND program. Those same 6 people are now represented in the Leadership Team and Core Faculty of the proposed Montana LEND: Kim McKearnan (OT), Julia Mazzarella (PT), Molly Kimmel (OT), Jennifer Banna (Family), Leah Meloy (SLP), and Krys Standley (Public Health). In the last 2+ years, they have been joined in their planning efforts by Luke Santore (lived experience as a person with a disability), Drew Babcock (curriculum design and Amskapi Pikuni (Blackfeet) descendant), and McKenzie Javorka (lead evaluator). Each person brings a wealth of knowledge, skills, and experience that has led to a program proposal that is truly interdisciplinary and reflective of the needs of Montanans with autism and other developmental disabilities. Each person listed above will dedicate between .05 - .25 (except for PD, Kim McKearnan, will give .30 - .40) FTE in support of advancing the mission of Montana LEND. This allotment of time has been carefully constructed to account for an individual's role with Montana LEND and their availability due to other project demands. As the program progresses, evaluations and feedback will lead to adjustments in FTE as needed. Additional Core and Affiliate Faculty have been recruited to assist once the program is funded. They reflect additional partners in LEND disciplines who bring great enthusiasm for the program goals and expertise in their fields. Each of them is dedicated to advancing Montana's healthcare and education workforce and supporting the next generation of clinical providers.

Project Director/Principal Investigator*

The Project Director provides leadership, coordination, and operational oversight. Working collaboratively with program leadership and interdisciplinary faculty, the Project Director shares responsibility for planning, designing, implementing, coordinating, and evaluating all aspects of the program to ensure high-quality education, training, and service delivery focused on autism and other developmental disabilities.

Key Responsibilities

Provide leadership for and facilitate collaboration in the strategic planning, design, implementation, and ongoing evaluation of the Montana LEND Program; Ensure grant funds are applied appropriately per the established budget and program plan; Assign appropriate roles and committee seats to Montana LEND faculty and staff; Ensure effective supervision and quality of all discipline-specific training and service activities; Work collaboratively with the assigned federal project officer to ensure that the program is being delivered as outlined and oversee the budget; Collaborate in the recruitment, selection, and evaluation of Montana LEND trainees, including developing and applying discipline-specific selection criteria; Foster collaboration and communication among professional colleagues, academic partners, clinical sites, and discipline-specific counterparts; Strengthen interdisciplinary partnerships to support integrated education, clinical practice, leadership development, and community engagement; Participate in program evaluation and continuous quality improvement activities; Contribute to program reporting, accreditation, and compliance activities; Represent Montana LEND within professional, academic, and community settings to promote interdisciplinary leadership and improve services; Attend LEND director meetings or conferences

Required Qualificatons

Must be a board-certified pediatrician or hold a doctorate in one of the health professions that are included in the LEND program; Minimum of 5 years of experience working with children with autism and other developmental disabilities; Must commit a minimum of 30% FTE to the leadership of the LEND program.

*Note: The co-project director will share these responsibilities

Core Faculty

The Core Faculty member collaboratively participates in the planning, implementation, and continuous improvement of MT LEND. The Core Faculty member ensures high-quality training, mentorship, and clinical education while promoting person- and family-centered practices for people with autism and other developmental disabilities. Each Core Faculty member will also participate in at least one sub-committee.

Key Responsibilities

Collaborate to plan, design, implement, coordinate, and evaluate the program; Provide leadership and oversight for the training and service components within their discipline, with emphasis on autism and developmental disabilities; Participate in the recruitment and selection of Montana LEND trainees; Serve as a mentor to short-, medium, and long-term trainees within their discipline and across related disciplines as appropriate; Foster interdisciplinary education and collaboration by supporting trainee learning, professional development, and leadership growth; Maintain effective communication and coordination with professional colleagues, academic partners, clinical departments, and state and community agencies; Contribute to program evaluation and quality improvement activities; Participate on at least one MT LEND committee (curriculum, outreach, short- and medium-term trainee engagement, etc.); Support the Montana LEND Program's mission.

Required Qualifications

Teaching, community-based service, and/or clinical experience providing person- and family-centered health care and related services to people with autism and other developmental disabilities; Demonstrated expertise within their professional discipline and commitment to interdisciplinary education and collaborative practice; Experience mentoring students and trainees; Ability to collaborate effectively with academic, clinical, and community partners; Commitment to advancing improved family-centered systems of care; Ability to commit a percentage of their FTE or equivalent stipend to the Montana LEND program as agreed upon with Montana LEND leadership

Data and Communications Coordinator

This to-be-hired role will work directly with the PD/PI to implement day-to-day operations. They will be primary point of contact for trainee communication, requiring extensive knowledge of Montana LEND components. They will create and oversee reporting structures and databases for collection of trainee hours, CE events, and TA activities. They will also work closely with the Evaluation Team.

Affiliate Faculty

Affiliate faculty members will complement and/or add expertise to core faculty skills and experiences. They will provide 1-2 didactic presentations each academic year in their area of expertise as well as assist with recruitment of trainees. Additionally, affiliate faculty members may provide input into the development of the Montana LEND curriculum specific to their profession or area of expertise.

Direct quotes from our letters of support (available upon request):

The Rural Institute for Inclusive Communities (RIIC) Consumer Advisory Council (CAC) offers our full support for the proposed funding from the Health Resources and Services Administration for a Leadership Education in Neurodevelopmental and Other Related Disabilities (LEND) program in the state of Montana.

The CAC is excited about the opportunity to support Montana LEND and is fully committed to ensuring its success. To support the program and enrich the trainee experience, our council will actively:

- Provide Direct Perspectives: Participate in panel presentations and educational sessions to share lived experiences with trainees.
- Facilitate Experiential Learning: Provide structured opportunities for trainees to observe CAC meetings, community initiatives, and advocacy activities.
- Enhance Recruitment: Assist in identifying and recruiting diverse trainees, particularly those with lived experience.
- Serve as Family Mentors: Guide and support trainees to foster a deeper understanding of family-centered care systems.

Community Medical Center has long been dedicated to the care of children and adults with developmental disabilities, and the training of healthcare professionals that provide this care. Our multidisciplinary teams, including physicians, nurses, therapists (speech, occupational, physical) will be directly involved in this partnership.

The most recent CSHS needs assessment found that Montana families of children with disabilities lacked access to providers in rural areas. We support the training and development of providers serving the Children and Youth with Special Health Care Needs population. CSHS looks forward to contributing to the MT LEND project.

The Developmental Disabilities Program (DDP) supports the Montana LEND program in building workforce capacity and fostering high-quality, interdisciplinary leadership across the state. Ultimately, this strengthens Montana's system of care, ensuring that individuals with developmental disabilities receive high-quality services even in the state's most rural and remote communities.

The Montana Council on Developmental Disabilities shares MT LEND's commitment to developing strong leaders, strengthening service systems, and increasing access to high-quality, person-centered supports for individuals with developmental disabilities and their families. MT LEND's focus on interdisciplinary leadership, family engagement, self-advocate participation, and workforce development aligns closely with the Council's mission to build capacity, advance systems change, and improve opportunities for meaningful community participation across Montana.

The Office of Public Instruction Montana Autism Education Project (MAEP) offers full support for the proposed funding from the Health Resources and Services Administration for a Leadership Education in Neurodevelopmental and Other Related Disabilities (LEND) program in the state of Montana.

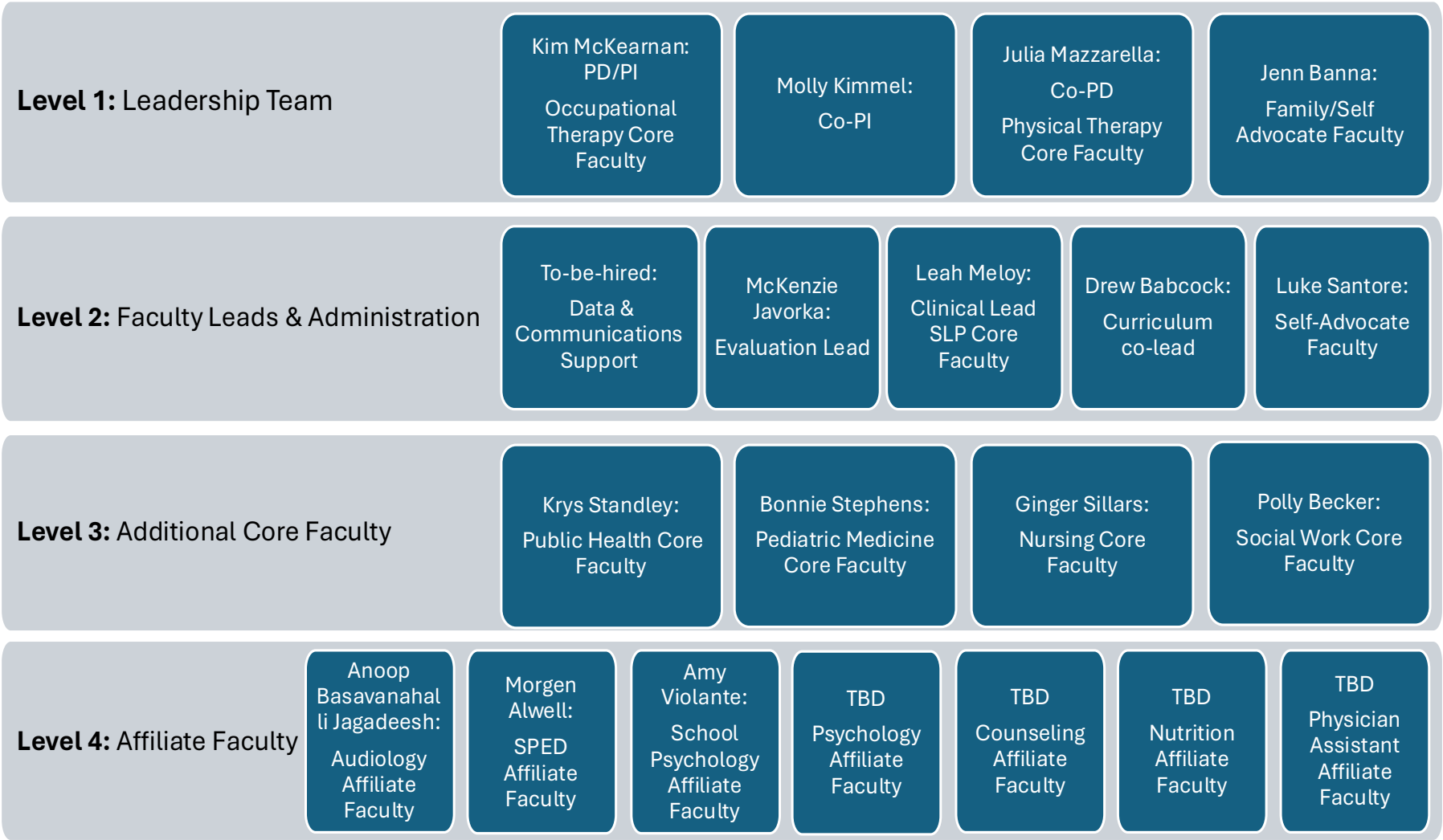
The MAEP supports these goals by connecting educators and school teams with research and resources, professional learning, coaching, and technical assistance that strengthen local capacity to support students receiving IDEA services. This work aligns closely with the proposed MT LEND's focus on workforce development, collaboration, and improving access to evidence-based services across Montana.

Shodair Children's Hospital strongly aligns with the goals of the Montana LEND program by providing interdisciplinary, family-centered care for children and adolescents with complex developmental, behavioral, mental health, and genetic needs. Through statewide clinical services, education, and collaboration across healthcare, schools, and community partners, Shodair helps develop the workforce and improve access to high-quality care for individuals with neurodevelopmental and related disabilities throughout Montana.

The mission of the MT LEND program is to improve the health-related quality of life of Montanans with neurodevelopmental and related disabilities by training interdisciplinary leaders, including graduate students, who work together to provide family-centered, culturally relevant, early detection and care across the lifespan while advocating for systems-level change. UM firmly believes that the MT LEND program will directly benefit both our students and our larger community. As an R1-Research designated university with a robust offering of healthcare and related degrees, a MT LEND is a natural fit on campus and will be well supported by the Rural Institute.

I, Dr. Schoffer Closson, and the YETI (Youth Engagement Through Intervention) social understanding program, MOSSAIC (Mentoring, Organization, and Social Support for Autism/All Inclusion on Campus) inclusion program, and the Meaningful Intervention for NeuroDiversity (MIND) Research &Advocacy Lab offer full support for the proposed funding from the Health Resources and Services Administration for a Leadership Education in Neurodevelopmental and Other Related Disabilities (LEND) program in the state of Montana.

Montana LEND Organizational Chart



Note. PI = Principal Investigator, PD = Project Director

Montana LEND Didactic Program Syllabus

Fall 2026 & Spring 2027

Course Description

This course is designed to meet the requirements of an interdisciplinary curriculum for both clinical and non-clinical trainees by targeting instruction on a comprehensive range of topics related to developmental disabilities, with a particular focus on autism spectrum disorder. Each topic fits into one of four distinct modules and culminates in final team leadership presentations. Each module has a designated course coordinator who will arrange a diverse set of presenters across subtopics. Each subtopic is taught as a four-hour course on Friday afternoons throughout the school year. The subtopics are explicitly designed to provide a breadth and depth of knowledge across practice settings, with themes of family-centered care and lived experiences prioritized across content areas. Reflective practice activities, such as reflection-in-action or reflection-on-action assignments, are embedded into each topic.

Course Format

All sessions are on Friday afternoons from 12:30-4:30 pm. Trainees and faculty are required to attend the first (Introduction) and last (Leadership Presentations) sessions in-person on the University of Montana (UM) campus. All other sessions are offered hybrid; trainees are encouraged to attend in-person on the UM campus as able, though a synchronous virtual option is available to accommodate placements across the vast state. Trainees need to inform the course director of their planned modality prior to each session to facilitate numbers for group discussions and activities.

Attendance

Trainees are allowed to miss two four-hour sessions total across semesters with prior approval from the module coordinator. Additional assignments may be provided to make up for missed topics to ensure all trainees receive comprehensive training across topics.

Mentored Leadership Project

Trainees will be assigned to small teams with a mentor. Teams will meet regularly across the course of the two semesters to develop a leadership project related to research, evaluation, or quality improvement in an institutional or community setting.

Orientation

Prior to the start of Friday afternoon didactics, all trainees and faculty will gather in Missoula for a Montana LEND Orientation for 1-2 days of content. Faculty will plan a mix of introductory activities, general course and module information, an overview of MCH competencies and Montana LEND leadership and clinical/community elements, and time for groups to begin working in their leadership teams. Perspective from a family and self-advocate panel will ground the trainees in the work of the year ahead. Pre-orientation readings and assessments will count towards LEND hours in the trainee's PEIP.

Course sessions

Module: Introduction and Theoretical Concepts

Coordinator: Krys Standley

Program Introduction and Orientation

Program overview, expectations, assignments; Introduction to interdisciplinary leadership; StrengthsFinder/personality assessments; Building cohort relationships and psychological safety; Reflective practice

Leadership Foundations

Leadership models and decision-making; Personal leadership identity and strengths; Adaptive and relational leadership approaches

Teamwork and Interdisciplinary Collaboration

Roles across disciplines; Effective teamwork and communication; Collaborative problem-solving and conflict management; Leadership projects creation and planning; Personal Interdisciplinary Experience Plan (PIEP) due

Social Determinants of Health

Social drivers impacting children and families; Structural inequities and access barriers; Community-based approaches to support

Models of Disability and Neurodiversity

Historical and contemporary disability models; Neurodiversity framework; Disability identity and strengths-based approaches; Leadership project proposal brief presentations

Autism and Developmental Disabilities Across the Lifespan

Autism/DD overview; Co-occurring conditions and lifespan considerations; Emerging topics and current issues

Module: Culturally and Linguistically Appropriate Services

Coordinator: Drew Babcock

Power, Privilege, and Intersectionality

Identity, disability identity, and systems of power; Intersectionality in healthcare and education; Reflective practice; Disability equity

Rurality, Montana Context, and Indigenous Perspectives

Rural systems and barriers to care; Workforce considerations: strengths and challenges; Montana-specific health and education systems; Indigenous health perspectives and community strengths

Cultural Safety and Inclusive Communication

Cultural humility and cultural safety; Ableism, microaggressions, and inclusive language; Culturally and linguistically appropriate services (CLAS)

Accessibility and Communication

Multilingualism; Motor and sensory differences; Health literacy; Working with interpreters

Panel Presentations on Lived Experiences

Community; Family members; Siblings; Past LEND trainees; Individuals with disabilities

Module: Systems Issues

Coordinator: Jenn Banna

Systems of Care for CYSHCN
Systems of care framework; Navigating fragmented systems; Collaboration between healthcare, schools, and community agencies
Medical Home and Care Coordination
Medical home principles; Care mapping and navigation tools; Transition planning and coordination strategies
Medicaid, Policy, and Funding Systems
Medicaid and reimbursement structures; Title V and Maternal Child Health systems; Policy development and advocacy basics
Disability Law and Educational Systems
IDEA, ADA, Section 504; Public school processes and supports; Parent Training and Information Centers
Transition to Adulthood
Healthcare transition planning; Adult systems and supports; Self-determination and independence
Data, Quality Improvement, and Outcomes Measurement
Using data for continuous quality improvement; Translating data into systems change; Individual and family outcome measurement; Program outcome measurement

Module: Clinical and Applied Issues

Coordinators: Julia Mazzarella and Leah Meloy
Screening and Diagnosis
Diagnostic procedures and screening tools; Interdisciplinary, multidisciplinary, and transdisciplinary teams and care coordination
Hard Discussions
Giving challenging (or new) news; Palliative care; Self-disclosure: sharing about diagnoses or genetic considerations
Intervention
Early intervention systems; Evidence-based and family-informed approaches; Interventions across disciplines
Genetics
What are genetics?; Screening and testing
Mental and Behavioral Health
Mental and behavioral health integration; Trauma-informed care; Family and caregiver mental health perspectives
Wellness and Secondary Traumatic Stress
Burnout, compassion fatigue, and secondary traumatic stress; Professional resilience and wellness practices; Sustainable leadership strategies
Evidence-Based Practice and Research Methods
Evidence-based practice and practice-based evidence; Research methods and critical appraisal; Identifying valid information and addressing bias
Innovation, Technology, and Emerging Practices
Telehealth and innovative care delivery; Emerging trends in autism/DD care; Future directions in interdisciplinary systems

Final Leadership Presentations and Reflection

Leadership presentations and discussion; Cohort reflection and celebration

MT LEND

Organization: University of Montana, Rural Institute for Inclusive Communities
Name of Project: MT LEND
Date Range: September 30, 2026 – June 30, 2031
Total Project Budget: $2,298,597

Salaries and Wages for University of Montana (UM) Personnel

MT LEND Core Faculty are paid by Full-time Equivalent (FTE) percentage for their time on the grant. There are currently 9 Core Faculty members who are already UM faculty/staff, 3 members of the Evaluation Team, and 1 person to-be-hired in as a Data and Communications Coordinator. Additional faculty costs are detailed in contract services for non-UM faculty. Affiliate faculty roles are described in the Project Narrative and are not compensated. As primary Principal Investigator/Program Director (PI/PD), Kimberly McKearnan will commit at least 30% of her time to the project each year. Each year includes a 3.5% cost of living increase for all staff salaries. FTEs are increased slightly in Years 1 and 2 to reflect extra staff time needed for new program development. Names, roles, and yearly FTE are described below with all costs outlined in detail in the budget document.

Roles and FTE/Salary Cost: $212,434 (Y1), $207,964 (Y2), $188,676 (Y3), $187,441 (Y4), $190,738 (Y5)

Name	Role	FTE
Kimberly McKearnan	PI/PD	.30 - .40
Molly Kimmel	Co-PI	.15 - .25
Julia Mazzarella	Co-PD	.18 - .20
Jennifer Banna	Family Faculty	.08 - .10
Luke Santore	People with lived experience Faculty	.05
Drew Babcock	Curriculum Co-chair	.08 - .10
Leah Meloy	Speech-Language Pathology Core Faculty	.08 - .10
Ginger Sillars	Nursing Core Faculty	.05
Krys Standley	Public Health Core Faculty	.08 - .10
McKenzie Javorka	Lead Evaluator	.15 - .20
Kaitlin Fertaly	Evaluation Oversight	.00 - .02
Elizabeth Sabbath	Evaluation Associate	.15 - .20
To-be-hired	Data & Communications Coordinator	.50

Fringe Benefits: $60,908 (Y1), $59,681 (Y2), $53,894 (Y3), $53,437 (Y4), $54,279 (Y5)

Fringe benefits are calculated at 25% for McKearnan, Sillars & Mazzarella, 28.9% for to-be-hired role, and 31.5% for all others

Health Insurance: $30,417 (Y1), $29,910 (Y2), $27,796 (Y3), $27,706 (Y4), $27,837 (Y5)

Health Insurance is pro-rated from $1,269/month for McKearnan, Sillars, & Mazzarella and $1,112/month for all others in Y1. Health Insurance costs escalate at 3% each year.

Total UM Personnel Breakdown

$303,759 (Y1), $297,555 (Y2), $270,366 (Y3), $268,584 (Y4), $272,854 (Y5)

Travel

Travel costs include local and long-distance travel for both MT LEND Faculty and Trainees as outlined below and detailed in the budget.

Local travel:

- Trainee travel for out-of-town trainees (locations TBD) to travel to Missoula for orientation, mid-year check-in, and final presentations/program wrap-up. Year 1 will include costs for 1 distance trainee, Year 2-3 for 2 distance trainees, and Years 4-5 for 3 distance trainees
- Travel costs for up to 4 outreach trips from Missoula to rural and tribal locations for faculty and trainee recruitment, clinical coordination, and community education. Locations TBA. Two trips will be day trips and two will be overnight trips. Participants will be a combination of up to 6 faculty and trainees.
- Travel for 1-2 LEND faculty to conduct outreach, recruitment, and programmatic meetings with community partners and stakeholders, locations TBA. Three trips will be day trips and 2 will be overnight.

Long-Distance Travel:

- Funding for PD/PI, 1 faculty, and 1 LTT to travel to DC for the AUCD conference in the fall in Y1-Y3. Reduced to PD/PI and 1 LTT in Years 4-5. Co-PI and other LEND faculty will attend based on funding from other sources (e.g. UCEDD Core Grant)
- Funding for 1 Faculty and 2 LTTs to travel to DC for the Disability Policy Seminar in Y1-Y2. Reduced to 1 Faculty and 1 LTT in Years 3-5. Co-PI and other LEND faculty will attend based on funding from other sources (e.g. UCEDD Core Grant)
- Funding for 1 PD to attend LEND recipient meeting in DC, timing TBA
- Funding for 1 PD to attend the Autism Cares recipient meeting in person in DC for Years 1, 2, and 4 per the NOFO

All Travel costs include estimated GSA rate for hotels, per diem, ground transport and/or airfare, and registration as needed.

Total Travel: $25,193 (Y1), $27,780 (Y2), $23,439 (Y3), $25,663 (Y4), $23,674 (Y5)

Participant Support

As a newly funded program, MT LEND is restricted to Tier 1 Funding and will support a minimum of 9 Long-Term Trainees (LTT) per funding year. At least 1 LTT will be a family advocate and 1 will be an individual with a disability. All LTTs will receive a stipend of $6,500 per training year ($58,500 total per year) at the completion of their 300 hours. Medium- and Short-Term Trainees will not receive financial compensation. In Years 3-5, an additional one LTT will be accepted as a Clinical Fellow with 20 hours/week of clinical experience supported by the MT LEND grant in the form of a $40,000 - $50,000 stipend). Trainees will be a mix of UM students, post-grads, community clinicians, and students from other universities.

Travel will also be covered for up to 5 clinical partners/short-term trainees to come to Missoula for 2-day ADOS or similar training. Each partner will have some travel coverage over the project period: $600/person (Y1), $500/person (Y2), $200/person (Y3 and Y4), and $40/person (Y5).

Total Participant Support: $61,500 (Y1) $61,000 (Y2), $104,500 (Years 3-4), $103,700 (Y5)

Materials & Supplies

Small Equipment

We will purchase necessary items to support hybrid instruction (360 degree camera, wireless microphones, dedicated laptop, etc.) We will also purchase clinical assessments like the ADOS-2 which includes kits and forms, as well as necessary clinic equipment like mats, positioning devices, sensory tools, and assistive technology. Most items will range in cost from $25 - $1,000 and nothing will be over $5,000.

Software

Software costs will offset the costs of the Evaluation Team's use of Monday.com. Non-UM trainees will also require purchase of a seat on Canvas for participation at $35/user.

Postage/Communications

Postage will be used to mail program materials to participants, as well as outreach materials to potential partners and continuing education recipients. Communications will cover any paid advertising of the program.

Total Materials & Supplies: $11,150 (Y1), $12,150 (Y2), $3,550 (Y3), $2,050 (Y4), $1,100 (Y5)

Meeting Cost (Facility/Room Rental)

For outreach, recruitment, and continuing education events outside of Missoula, this funding will cover the cost of renting a room at a local business as needed.

Total Facility/Room Rental: $50-300 (Years 1-5)

Contract and Consultant Services

Transcription

Rev.com rates are $2/minute. The Evaluation team expects $1,990 in transcription costs for focus groups and interviews each year.

Background Checks

Each trainee will be required to complete a background check to participate. This cost is approximately $80 per person.

Non-UM Faculty

MT LEND will enter into contracts with the following non-UM faculty members. Non-UM faculty member stipends were determined in relation to their salary and time identified on the LEND grant (10 hours/month). Additional non-UM faculty will be identified as the program progresses, accounting for the increase in costs.

Bonnie Stephens, Developmental Pediatrician Core Faculty, currently working as Chief Medical Officer and Medical Director of the Community Medical Center Neonatal intensive Care Unit in Missoula

Polly Becker, Social Work Core Faculty, currently employed by Community Medical Center in Missoula as a Licensed Clinical Social Worker working with developmental-behavioral pediatric clients.

Accessibility Services

Hybrid Instruction and continuing education opportunities need to be accessible for people with disabilities. MT LEND will contract as needed with CART providers and ASL interpreters. The Rural Institute also has translation services so that continuing education materials can be translated into other languages for widespread distribution.

ADOS-2 Training

A well-renowned ADOS trainer from eastern Washington will be contracted to provide once yearly training in this standardized Autism Assessment. We will pay for the costs of her training at $7,500/year.

Guest Speakers/Family Advocates

People with lived experience of disability will frequently engage with LEND trainees as guest speakers in the didactic program and as family mentors. These participants will receive compensation for their time through UM's contracted services process. Average payments will be $125 to 15-20 people.

Total Contract/Consultant Services: $28,210 (Y1), $31,210 (Y2-Y3), $32,210 (Y4-Y5)

Administrative or Indirect Costs: 8%

Total IDC: $29,505 (Y1), $29,520 (Y2), $26,309 (Y3), $26,305 (Y4), $26,391 (Y5)

Total Yearly Costs

$459,817 (Y1), $459,515 (Y2), $459,674 (Y3), $459,612 (Y4), $459,979 (Y5)

Project Total: $2,298,597